



**MINISTRY OF LABOUR AND
SOCIAL PROTECTION**

STATE DEPARTMENT FOR SOCIAL PROTECTION
AND SENIOR CITIZEN AFFAIRS

**COMMUNITY BASED
INCLUSIVE DEVELOPMENT**

TRAINING MANUAL

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Abbreviations & Acronyms

CBID	Community Based Inclusive Development
CBM	Christian Blind Mission
CBR	Community Based Rehabilitation
CHP	Community Health Promoters
CRPD	Convention on the Rights of Persons with Disabilities
ECD	Early Childhood Development
MDACs	Ministries Departments Agencies and Counties
OPDs	Organization of and for Persons with Disabilities
OVC	Orphans and Vulnerable Children
PBO	Public Benefit Organizations
PPT	PowerPoint Presentation
SDGs	Sustainable Development Goals
UN	United Nations
WHO	World Health Organization

FOREWORD



The Government of Kenya reaffirms its unwavering commitment to the empowerment and inclusion of persons with disabilities, their families, and communities. Guided by the principles of equity, dignity, and social justice, we recognize Community-Based Inclusive Development (CBID) as a transformative approach that aligns with our national development agenda and the global Sustainable Development Goals (SDGs). CBID offers a practical, participatory, and people-centered approach to achieving these goals at the grassroots level, ensuring that no one is left behind in our pursuit of inclusive and sustainable development.

This CBID Training Manual marks a significant milestone in our collective journey toward building inclusive, empowered, and resilient communities across Kenya. Developed in response to the growing need for locally grounded approaches to disability inclusion, it draws on global best practices, national policy frameworks, and most importantly the lived experiences of individuals with disabilities and their communities. It reflects our shared commitment to ensuring that persons with disabilities are not only included in development processes but are central to shaping them.

Designed as a comprehensive resource for Community Social Development Promoters, educators, community volunteers, and all stakeholders engaged in CBID implementation, the manual aims to build capacity, enhance coordination, and equip practitioners with the knowledge, tools, and strategies necessary to support persons with disabilities in realizing their full potential within inclusive communities. Anchored in the principles of human rights, empowerment, and active participation, the manual addresses critical areas including health, education, livelihoods, social inclusion, and community empowerment. It is informed by both local experiences and international standards, offering a practical framework for translating inclusive development principles into everyday practice.

The manual also outlines key areas of focus for various agencies and calls for collaborative effort and coordinated action to achieve collective impact. We urge all actors, government agencies, civil society, development partners, and community leaders to embrace this agenda and play their part in implementing the CBID Programme. Together, we must integrate CBID into our social protection systems and development programmes to ensure continuity, sustainability, and systemic change.

CBID practitioners occupy a critical role in shaping communities that are equitable, resilient, and responsive to the diverse needs and aspirations of all individuals, including persons with disabilities. This manual serves as both a training resource and a strategic instrument for advancing advocacy, fostering innovation, and promoting inclusive development. We call upon all stakeholders to actively apply its principles, collaborate across sectors, and champion inclusive practices that drive sustainable change.

A handwritten signature in blue ink, appearing to read 'A. Mutua', with a stylized flourish at the end.

Dr. Alfred N. Mutua, E.G.H
Cabinet Secretary
Ministry of Labour and Social Protection

ACKNOWLEDGEMENT



The development of the *Community-Based Inclusive Development (CBID) Training Manual* was a truly collaborative effort, made possible through the dedication, expertise, and commitment of numerous individuals and institutions.

We extend our heartfelt gratitude to the various Ministries, Departments, and Agencies; development partners; Organizations of and for Persons with Disabilities; and all stakeholders whose unwavering support made this manual possible.

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We greatly appreciate the technical and financial support provided by Christian Blind Mission and The Action Foundation (TAF). We also appreciate the meaningful input from our partners, including the United Disabled Persons of Kenya (UDPK), Differently Talented Society of Kenya (DTSK), Caucus of Disability Rights and Advocacy (CDRA), Deaf Empowerment Kenya (DEK), Kenya Society for the Blind (KSB), and the Kenya Association of the Intellectually Handicapped (KAIH). Your partnership, collaboration and commitment were instrumental in shaping the content and direction of this manual.

Our sincere appreciation goes to the communities and persons with disabilities who generously shared their lived experiences. Their voices brought authenticity, depth, and relevance to this resource, ensuring that it reflects the realities, strengths, and aspirations of the communities it seeks to serve. While it is not possible to mention every contributor by name, we extend our deepest appreciation to all who supported this initiative. Your collective effort and unwavering dedication have made this manual a vital tool for advancing inclusive community development across Kenya.

Last but not least, we thank the editors, reviewers, and administrative teams whose behind-the-scenes efforts brought this publication to life.



Joseph M. Motari, CBS

Principal Secretary

State Department for Social Protection and Senior Citizen Affairs

INTRODUCTION

Community-Based Rehabilitation (CBR) was introduced in 1978, after the Alma-Ata Declaration of Health for All by WHO. According to the World Health Organization (WHO), CBR is a strategy for rehabilitation, equalization of opportunities, poverty reduction, and social inclusion of persons with disabilities. It empowers persons with disabilities to access health, education, livelihood, and social opportunities using locally available resources. It seeks to ensure their inclusion and equal opportunities by promoting rehabilitation, equalization of opportunities, and social integration all within the community setting. CBR is grounded in the belief that disability inclusion is a shared community responsibility, not just a specialized service provided by institutions.

To ensure that persons with disabilities have an equal choice to live independently in the community and that they have access to quality and affordable support services and assistive devices as implied in the Convention on the Rights of Persons with Disabilities (CRPD), it is necessary for the Government to invest further in community-led interventions that are disability specific.

Over the years, CBR has evolved into Community Based Inclusive Development (CBID) targeting and involving persons with disabilities, their families and their respective organizations as primary stakeholders to improve the quality of life and ensure their empowerment, participation, and inclusion in society.

Community-Based Inclusive Development (CBID) is a multi-sectoral approach working to improve the equalization of opportunities and social inclusion of persons with disabilities while combating the perpetual cycle of poverty and disability. It is implemented through the combined efforts of persons with disabilities, their families, and communities through the support of the government, Organizations of and for Persons with Disabilities (OPDs) and other development partners.

The concept of community-based inclusive development hinges on three aspects, according to which all development initiatives should: (a) be community-based and inclusive of persons with disabilities; (b) be centered on persons with disabilities and the community; and (c) address the needs of persons with disabilities, their families and communities.

There is a clear indication that persons with disabilities to date still face great challenges of accessing services in the community. Stigma and discrimination is still rampant in certain parts of the country. Some of the children with disabilities are institutionalized (i) because of the stigma a family that has a child with disability faces (ii) the lack of resources to care for this child within the community (iii) the lack of available services such as education, health and recreation that are specific to the needs of the children or youth with disabilities and these are key areas that can be addressed through Community Based Rehabilitation.

The Government, to address issues of reintegration of children with disabilities back into the community, has rolled out the National Positive Parenting Programme and the National Care Reform Strategy all aimed at ensuring families and communities are strengthened and able to care for vulnerable populations inclusive of children and adults with disabilities.

Overall objective

To provide a standard training tool that enhances the capacity of trainers for efficient and effective implementation of CBID initiatives with an aim of facilitating community-based inclusive development by, for and with persons with disabilities and their communities.

Specific Objectives

- To increase community awareness and participation in disability programs.
- To equip trainers with knowledge and skills for effective implementation of CBID
- To demonstrate practical approaches for applying key components of CBID
- To enhance coordination and collaboration of stakeholders within the CBID space
- To provide user-friendly training content for families and communities on disability inclusion.

Target users of the manual

The training manual will be used by:

1. Social Development Resource.
2. Organizations of and for persons with disabilities.
3. Community groups.
4. Ministries Departments, Agencies and Counties.
5. Development agencies .
6. Civil society.
7. Stakeholders.
8. Community.

Training sessions

The training sessions comprise of six modules, including an introduction module, and the five substantive modules.

MODULES

The training manual has 6 modules

1. **Introduction to CBID**
2. **Health**
3. **Education**
4. **Livelihood**
5. **Social**
6. **Empowerment**

MODULE 1 : INTRODUCTION TO COMMUNITY-BASED INCLUSIVE DEVELOPMENT (CBID)

Module description

This module will cover basic introduction to CBID, outline the components and explain the benefits of CBID

Module Outcomes:

By the end of the session the learners will be able to: -

- Define CBID
- State the components of CBID
- State the benefits of CBID.

Module Topics

SN	Topic Names	Duration (Minutes)
1.1	Introduction to CBID	40
1.2	Legal Frameworks	60
	Total	100

TOPIC 1.1: INTRODUCTION TO CBID

Methodology:	Flip Charts, Marker pen, LCD, Masking tape
Time:	100 Minutes
Facilitator Instruction:	
1. Begin with an icebreaker question: "Who knows what CBID is?" (5 min). 2. Present the definition of CBID (5 min). 3. Divide learners into small groups (5–8 members). Ask each group to share examples of CBID initiatives in the community and their benefits (15 min). 4. Present the benefits of CBID to persons with disabilities and the community (15 min)	

Community Based Inclusive Development

CBID is a strategy to promote inclusion, rights and equal opportunities for persons with disabilities to ensure inclusive development.

1.1.1 Components of CBID

The CBID components include;

1. **Health;** Health Promotion; Prevention; Medical care; Rehabilitation; Assistive devices
2. **Education;** Early childhood care and education; Primary, Secondary and higher education, non-formal and lifelong learning

3. **Livelihood**; Skill development; Self-employment; Wage employment; Financial services; Social protection
4. **Social**; Personal assistance; Relationship, Marriage and family; Arts and Culture; Recreation, leisure and Sports; Access to justice
5. **Empowerment**; Advocacy and communication; Community mobilization; Political participation; Self-help groups; Organization of and for persons with disabilities

1.1.2 Benefits of CBID to persons with disabilities

- o Addressing impairments, improving functioning and independence and promoting equal participation.
- o Empowering persons with disabilities to make informed decisions, attain their goals and understand the individual rights.
- o Enhancing the quality of life of persons with disabilities and their caregivers by addressing basic needs gaps.
- o Encouraging and supporting persons with disabilities and their families to contribute their skills and resources to community development.
- o Challenging and addressing barriers that exclude persons with disabilities and their families from social roles and activities.
- o Enhancing self-confidence and self-esteem, leading to a positive appreciation and identification with their unique qualities.

1.1.3 Benefits of CBID to the community

- o Builds awareness of diversity within the community including diversity in function of persons with disabilities, older persons, youth, and children.
- o Fosters more positive attitudes towards persons with disabilities and creates greater understanding of disability, rights and the importance of equity so that no one is left behind when community development activities are initiated .
- o Promotes inclusion of all people particularly persons with disabilities in local decision-making governance and resource allocation.
- o Communities recognize that persons with disabilities are valued members, in which they can make positive and meaningful contributions to the community.
- o Local government authorities respond to the needs of persons with disabilities and their family members and provide effective social support and services where required.

TOPIC 1.2: LEGAL FRAMEWORKS

Methodology:	Flip Charts, Marker pen, LCD, Masking tape
Time:	60 Minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: "What is a legal framework?" (5 min). 2. Present the definition of Legal Framework (5 min). 3. Divide learners into 4 groups. Ask each group to discuss the different legal frameworks at the regional context, national context and the policy frameworks and make presentations. 4. Share examples of CBID initiatives in the community and their benefits (35 min) 5. Present different legal frameworks that address persons with disabilities (15 min)	

A legal framework refers to the system of laws, regulations, policies, and legal principles that govern a particular area of activity or organization. It provides the structure within which legal rights and obligations are defined, enforced, and interpreted.

United Nations Convention on the Rights of Persons with disabilities (UNCRPD): Kenya signed the CRPD in 2007 and ratified it in 2008. The purpose of the Convention is to promote, protect and ensure the full and equal enjoyment of all human rights and fundamental freedoms by all Persons with disabilities and to promote respect for their inherent dignity. Specific articles most relevant to CBID components include, Article 8 (Awareness-raising), Article 12 (Equal recognition before the law), Article 13 (Access to justice), Article 19 (Living independently and being included in the community), Article 23 (Respect for home and the family), Article 24 (Education), Article 27 (Work and employment), Article 28 (Adequate standard of living and social protection), Article 29 (Participation in political and public life), Article 30 (Participation in cultural life, recreation, leisure and sport)

Sustainable Development Goals (SDGs): The 2030 agenda for Sustainable Development includes 17 goals built on the principle of 'Leave no one behind'. The agenda holds a deep promise for persons with disabilities everywhere. All the 17 goals are relevant for the inclusion agenda of development of persons with disabilities; however, the following make specific reference to disability.

Goal 1 (End poverty in all its forms everywhere), 4 (Ensure inclusive and equitable quality education and promote lifelong learning opportunities for all), 8 (Promote sustained, inclusive and sustainable economic growth, full and productive employment and decent work for all), Goal 10.2 (By 2030, empower and promote the social, economic and political inclusion of all, irrespective of age, sex, disability, race, ethnicity, origin, religion or economic or other status), 11 (Make cities and human settlements inclusive, safe, resilient and sustainable) and 17 (Strengthen the means of implementation and revitalize the Global Partnership for Sustainable Development).

REGIONAL CONTEXT

Protocol to the African Charter on Human Rights and Peoples Rights on the Rights of Persons with disabilities in Africa: The African Disability protocol was adopted by the African Union (AU) assembly on 29th January 2018. Kenya ratified the protocol in February 2022. The primary objective of the Protocol is to promote, safeguard and ensure the complete and equal exercise of all human and people's rights for individuals with disabilities in Africa, as well as to ensure respect for their inherent dignity. The protocol complements the UN Convention on the Rights of Persons with disabilities (UNCRPD) by addressing the rights of persons with disabilities from an African perspective, considering the lived realities of individuals with disabilities on the continent while maintaining the core values and principles as set forth in the UNCRPD.

The following articles in the protocol are relevant to CBID; Article 5 (Non-discrimination), Article 7 (Equal recognition before the law), Article 13 (Right to access justice), Article 14 (Right to live in a community), Article 16 (Right to education), Article 17 (Right to health), Article 18 (Habilitation and Rehabilitation), Article 19 (Right to work), Article 22 (Self representation), Article 23 (Right to freedom of expression and opinion), Article 24 (Right to information).

NATIONAL CONTEXT

Constitution of Kenya (2010): The Constitution of Kenya, 2010, confers upon its citizens, including Persons with Disabilities, the right to the enjoyment of all human rights and fundamental freedoms. The basis of these protections is spelt out in the national values and principles of governance. The following tabulated Articles are relevant to disability inclusion:

Article	Content
7(3)(b)	Promotes development and use of indigenous languages, Kenya Sign Language, Braille, and other communication formats and technologies accessible to persons with disabilities.
10	Binds all State organs, State officers, public officers, and all persons applying or interpreting the Constitution to make or implement public policy decisions that among other things promote non-discrimination. It also seeks to minimize barriers to equalization of opportunities for persons with disabilities in all aspects including socio-cultural, economic, and political life.
11	Culture (1) This Constitution recognizes culture as the foundation of the nation and as the cumulative civilization of the Kenyan people and nation. (2) The State shall (a) promote all forms of national and cultural expression through literature, the arts, traditional celebrations, science, communication, information, mass media, publications, libraries and other cultural heritage.
27	Further guarantees the right to equality and freedom from discrimination and expressly prohibits discrimination on the grounds of disability.
41	Labour relations (1) Every person has the right to fair labour practices.
43	Economic and social rights (1) Every person has the right— (a) to the highest attainable standard of health, which includes the right to health care services, including reproductive health care; (b) to accessible and adequate housing, and to reasonable standards of sanitation; (c) to be free from hunger, and to have adequate food of acceptable quality; (d) to clean and safe water in adequate quantities; (e) to social security; and (f) to education.
45	Family (1) The family is the natural and fundamental unit of society and the necessary basis of social order and shall enjoy the recognition and protection of the State. (2) Every adult has the right to marry a person of the opposite sex, based on the free consent of the parties. (3) Parties to a marriage are entitled to equal rights at the time of the marriage, during the marriage and at the dissolution of the marriage. (4) Parliament shall enact legislation that recognizes— (a) marriages concluded under any tradition, or system of religious, personal or family law; and (b) any system of personal and family law under any tradition, or adhered to by persons professing a particular religion, to the extent that any such marriages or systems of law are consistent with this Constitution.
48	Access to justice The State shall ensure access to justice for all persons and, if any fee is required, it shall be reasonable and shall not impede access to justice.

Article	Content
53(1)	Children (1) Every child has the right— (a) to a name and nationality from birth; (b) to free and compulsory basic education; (c) to basic nutrition, shelter and health care; (d) to be protected from abuse, neglect, harmful cultural practices, all forms of violence, inhuman treatment and punishment, and hazardous or exploitative labour; (e) to parental care and protection, which includes equal responsibility of the mother and father to provide for the child, whether they are married to each other or not; and (f) not to be detained, except as a measure of last resort, and when detained, to be held— (i) for the shortest appropriate period of time; and (ii) separate from adults and in conditions that take account of the child's sex and age.
53(2)	A child's best interests are of paramount importance in every matter concerning the child.
54	Outlines the entitlements to persons with disabilities, a. To be treated with dignity and respect and to be addressed and referred to in a manner that is not demeaning, b. To access educational institutions and facilities for persons with disabilities that are integrated into society to the extent compatible with the interests of the person, c. To reasonable access to all places, public transport, and information; to use Sign language, Braille, or other appropriate means of communication; d. To access materials and devices to overcome constraints arising from the person's disability. The State shall ensure the progressive implementation of the principle that at least five percent of the members of the public in elective and appointive bodies are persons with disabilities.

Persons with Disabilities Act, 2025: This is a comprehensive law that promotes and protects the rights and equalization of opportunities for persons with disabilities. It covers rights, rehabilitation, and equal opportunities for Persons with disabilities and aligns with the Constitution of Kenya 2010 and UNCRPD. The act has various provisions, rights, and privileges relating to health, education, disability mainstreaming, awareness creation, and accessibility, among others. It establishes the National Council for Persons with disabilities (NCPWD) which among other role supervises the implementation of the Act by other relevant MDAs and County Government that promotes and protects equalizations of opportunity.

The Children's Act, 2022: It aims to safeguard the rights and welfare of children. It provides a robust legal framework for protecting children from abuse, neglect, and exploitation, ensuring their access to education, health, and essential services. The Act also establishes provisions for parental responsibility, alternative care, and the administration of children's services.

Basic Education Act, 2013: It mandates the establishment of special and integrated schools for learners with disabilities, ensuring they can access education in appropriate settings, whether separate or within mainstream society.

The Early Childhood Education Act, 2021: It guarantees all children, including those with disabilities, the right to early childhood education without discrimination. It requires county governments to provide accessible ECE, identify and assess children with disabilities, and ensure no actions hinder their access. These provisions promote inclusive and equitable education for every child.

The Technical and Vocational Education and Training Act, 2013: The TVET Act 2013 in Kenya promotes disability inclusion by requiring institutions to mainstream special needs, ensure accessibility (physical and instructional), provide reasonable accommodation, offer guidance/counselling, and implement policies that align with the Rights and Protection of Persons with Disabilities

Employment Act, 2007: Contains elaborate provisions aimed at protecting persons from discrimination in employment which explicitly prohibits an employer from discriminating directly or indirectly against an employee or prospective employee or harassing an employee or prospective employee on grounds of disability.

Work, Injury, Benefit Act, 2007: Provides compensation for work-related injuries and diseases, including disability, through different benefit categories. The Act outlines the rights of employees to receive benefits in case of work-related incidents and establishes the framework for claims and compensation.

The Public Procurement and Asset Disposal Act, 2015: Provides for the national and county governments to facilitate affirmative action for disadvantaged groups in accordance with the Constitution and advance their participation in the procurement process. The Act supports Access to Government Procurement Opportunities (AGPO) which facilitates the youth, women, and persons with disabilities-owned enterprises to be able to participate in government procurement. It mandates at least 30% of all government procurement opportunities to be set aside for women, youth and persons with disabilities.

The Community Groups Registration Act, 2022: It provides a structure for the management of groups' registration, coordination, mobilization, and control to legitimize and sustain them.

Social Protection Act, 2025: It provides support for vulnerable groups including orphans, older persons, and persons with disabilities; cushion people from social and economic risks; and promote dignity and well-being. Through this Act persons with disabilities will receive respite services, rehabilitative services and psychosocial support among others.

Social Health Insurance Act, 2023: It covers persons with disabilities who are wholly dependent on and living with contributors. It expands benefits for persons with disabilities in terms of coverage for assistive devices, covers family wide protection, inclusion of long term and rehabilitation.

POLICY FRAMEWORKS

Kenya Vision 2030 blueprint: Provides for specific strategies to improving access to all disadvantaged groups (e.g., Business opportunities, health, education, housing and justice) and minimizing vulnerabilities through prohibition of retrogressive practices (e.g., female genital mutilation and child labour) and by upscaling the training for person with disabilities and special needs. It also recognizes disability mainstreaming as one of the flagship projects under the Social Pillar.

Persons with Disabilities National Policy 2024: The purpose of this policy is to inform and provide guidelines for planning budgeting, implementation, monitoring and evaluation of policies, legislations, programmes and specific measures for mainstreaming disability and empowering persons with disabilities.

Kenya Social Protection Policy 2024: The Policy presents the Government's vision of creating an all-inclusive society through the provision of sustainable mechanisms for the protection of persons living in situations of extreme poverty and vulnerability. The Policy identifies orphans and vulnerable children, persons with disabilities, older persons, the urban poor living in slum areas, people living in the streets in urban areas, people living with dilapidating illness, the internally displaced persons, among others as vulnerable and qualifying for social protection.

Sector Policy for Learners and Trainees with Disabilities: To provide a robust sector policy framework for provision of education and training for learners and trainees with disabilities.

National Policy on Gender and Development 2019: The overall goal of this policy is to achieve gender equality by creating a just society where women, men, boys and girls have equal access to opportunities in the political, economic, cultural, and social spheres of life.

Kenya Universal Health Coverage (UHC) Policy 2020-2030: to ensure all Kenyans have access to essential quality health services without suffering financial hardship.

MODULE 2 : HEALTH COMPONENT

Module description

This module will cover the general concept of health, promotion, prevention, medical care, rehabilitation and assistive technologies

Module outcomes:

By the end of the session the learners should be able to:

- Define the concept of health.
- Describe the legal frameworks on health and disability.
- Explain the concept of inclusive health.
- Identify and address barriers to healthcare services for persons with disabilities.

Module Topics

SN	Topic Names	Duration (Minutes)
2.1	Define the concept of health.	165
2.2	Health Promotion	115
2.3	Prevention	70
2.4	Medical care	70
2.5	Rehabilitation and Habilitation	85
2.6	Assistive Technology	45
	Total	550

TOPIC 2.1: THE CONCEPT OF HEALTH

Methodology:	Interactive lecture (PPT Presentation) + small group discussion.
Time:	30 minutes.
Facilitator Instructions:	
1. Begin with an icebreaker question: "What does being healthy mean to you?" (5 min). 2. Present the WHO definition of health and briefly explain the 3 components: physical, mental, social well-being (10 min). 3. Divide learners into small groups (5–8 members). Ask each group to share examples of how health is more than just the absence of disease (e.g., stress-free life, supportive relationships) (10 min). 4. Summarize key points, linking learners' examples back to the WHO holistic definition (5 min).	

Definition: According to the World Health Organization (WHO), health is *"a state of complete physical, mental and social well-being, and not merely the absence of disease or infirmity."*

Health is a valuable resource that enables people to lead individually, socially and economically productive lives, providing them with the freedom to work, learn and engage actively in family and community life.

Health includes:

- **Physical well-being:** The body is strong, free from disease, and functioning well.
- **Mental well-being:** The ability to cope with stress, think clearly, and maintain good relationships.
- **Social well-being:** Being able to participate in society, interact with others, and live with dignity.

Key Message: Health is therefore holistic, not just about treating sickness.

2.1.1 Legal Frameworks on Health and Disability

Methodology:	Guided discussion + case study analysis.
Time:	40 minutes.
Facilitator Instructions:	
1. Trainer introduces the topic and gives the existing legal frameworks. 2. Present a short overview of key international and national frameworks (10 min). 3. Give learners a case scenario: e.g., “A child with disability is denied admission at a health facility because of lack of wheelchair access.” Ask: <i>Which legal framework protects this child’s rights?</i> (15 min). ➤ Facilitate discussion in plenary, encouraging learners to link specific frameworks (CRPD, Constitution of Kenya, Disability Act, etc.) to real situations (10 min). 4. Wrap up with a summary chart showing each framework and its role (5 min).	

International frameworks:

- United Nations Convention on the Rights of Persons with Disabilities (CRPD) – promotes the right of persons with disabilities to health without discrimination.
- Sustainable Development Goals (SDGs) – Goal 3 aims for health and well-being for all.
- WHO- We have WHO CBR guidelines 2010

National frameworks (Kenya):

- The Constitution of Kenya (2010) – Article 43 guarantees the right to the highest attainable standard of health.
- Persons with Disabilities Act (2025) – provides for equal access to health services.
- Health Act (2017) – emphasizes equity, quality, and accessibility in healthcare.

Take away: A child with a disability being denied admission to a health facility due to lack of wheelchair access is a violation of their right to health, as protected by several legal frameworks including the UN Convention on the Rights of Persons with Disabilities, Kenya’s Constitution of 2010, the Persons with Disabilities Act (2025), and the Health Act (2017).

2.1.2 The Concept of Inclusive Health

Methodology:	Demonstration + role play + brainstorming.
Time:	35 minutes.
Facilitator Instructions:	
1. Start with a short story/example (e.g., a clinic that welcomes everyone vs. one that excludes persons with disabilities) (5 min). 2. Ask learners to brainstorm in pairs: What would an inclusive health facility look like? (10 min). 3. Select 2 - 3 volunteers to do a role play: one as a health worker, another as a patient with disability. Show inclusive vs. non-inclusive interaction (10 min). 4. Facilitate a discussion to highlight inclusive practices: accessibility, affordability, trained staff, etc. (5 min). 5. Conclude with a take-home message: inclusive health = equity in action (5 min).	

Inclusive health

The concept of inclusive health is now being promoted by CBID Programmes to ensure health systems recognize and accommodate the needs of persons with disabilities in their policies, planning and services delivery. It means that all individuals can access health care irrespective of impairment, gender, age, color, race, religion and socioeconomic status.

Inclusive health means everyone has equal access to health services, regardless of age, gender, disability, or social status.

It focuses on:

- o Removing barriers that prevent people, especially persons with disabilities, from using health services.
- o Ensuring services are accessible, affordable, acceptable, and of good quality.
- o Training health workers to understand and respond to diverse needs.

Example: A clinic with wheelchair access, sign language interpreters, and patient information in braille.

Take away; Inclusive health ensures that all individuals can access health services without discrimination by actively removing barriers and accommodating the diverse needs of everyone, regardless of their background or disability.

2.1.3 Barriers to Healthcare Services for Persons with Disabilities

Methodology:	Group work + problem-solving exercise.
Time:	40 minutes.
Facilitator Instructions:	

1. Present the 5 categories of barriers (physical, communication, attitudinal, financial, policy/system) with brief examples (10 min).
2. Divide learners into 5 groups; assign each group one barrier type. Ask them to:
 - Identify 2 examples of this barrier in their community.
 - Suggest 1 possible solution. (15 min).
3. Each group presents back in plenary (10 min).
4. The facilitator summarizes solutions into a visual chart

Common Barriers to Health-Care Services for Persons with Disabilities and Their Families

1. Physical and Geographical Barriers

Persons with disabilities often face difficulties reaching and using health facilities due to:

- Inaccessible transport (buses, trains, air, and water transport).
- Buildings without standardized ramps, lifts with braille or voice prompts, safe walkways, inclusive signage, or accessible washrooms.
- Lack of supportive spaces for persons with neurodiversity.
- Health facilities without disability-friendly equipment (e.g., adjustable beds, user-friendly MRI/CT scan machines, dental chairs).
- Sensory overload challenges for persons with sensory sensitivities, who may need orientation or simulations before procedures.
- Rural areas with few facilities, long distances to urban centers, and limited specialized services.

Promote inclusive infrastructure, advocate for accessible transport, provide disability-friendly medical equipment, train health workers, and strengthen rural health systems to reduce dependence on urban facilities.

2. Communication and Information Barriers

Access to health care is limited when communication and health information are not inclusive.

Examples include:

- Lack of sign language interpreters in facilities.
- Health workers with little or no training in Kenyan Sign Language.
- Absence of braille, pictorial, or easy-to-read materials for persons with visual, intellectual, or learning disabilities.
- Health messages in press conferences, pamphlets, or media without sign language, subtitles, or simplified versions.

This leaves many unable to explain symptoms, understand diagnoses, or follow treatment.

Solutions: Addressing Communication and Information Barriers

- Train and deploy Kenyan Sign Language interpreters in health facilities, starting with referral and county hospitals.
- Train health workers in basic Kenyan Sign Language for everyday communication.
- Develop and distribute Braille materials, pictorial guides, and easy-to-read formats for health education.
- Ensure press conferences and public health messages include sign language interpretation and subtitles.

- Use community health workers to explain health information in accessible formats at household and community level.
- Leverage mobile technology (SMS, WhatsApp, interactive voice response) to deliver health information in multiple accessible formats.

3. Attitudinal Barriers

Negative attitudes, prejudice, and lack of knowledge among health workers remain a major barrier. Examples:

- A nurse assuming a woman with disability cannot get pregnant, and therefore denying her family planning advice.
- Doctors lacking training in assistive device fitting, forcing referrals to distant facilities.
- Limited appreciation of disability cultures, such as Deaf culture.

Past negative experiences (e.g., being ignored, mishandled, or carried physically into a facility due to inaccessibility) may also discourage persons with disabilities from returning.

Solution: Addressing Attitudinal Barriers

- Integrate disability awareness and inclusion training into pre-service and in-service curricula for all health workers.
- Promote peer-to-peer learning by including persons with disabilities as trainers in health worker programs.
- Establish disability focal persons in health facilities to champion inclusive service delivery.
- Encourage health facilities to adopt a “disability-friendly charter” that commits staff to respectful, inclusive care.
- Use positive role models and testimonials from persons with disabilities to challenge myths and stereotypes.
- Introduce feedback and complaint mechanisms (anonymous boxes, hotlines) so patients can safely report discrimination or mistreatment.

4. Knowledge Barriers

- Persons with disabilities and their families may have limited knowledge of their rights and available services.
- Some may wrongly assume they are not entitled to services such as immunization or therapy.
- Limited awareness of services (e.g., not knowing that hearing aids are available at a county hospital).
- Negative beliefs (e.g., thinking arthritis pain is “normal aging” or that disability is a curse, so medical help is useless).

Solutions: Addressing Knowledge Barriers

- Conduct awareness campaigns to educate persons with disabilities and families on their rights and available services.
- Disseminate user-friendly guides (e.g., posters, community talks, radio programs) about available rehabilitation and assistive technology services.
- Partner with Disabled Persons Organizations (DPOs) to reach communities with accurate health information.
- Encourage peer support groups where persons with disabilities can share information and experiences.
- Integrate disability rights and health education into community health promoters (CHP) activities.

5. Financial and Economic Barriers

- o Out-of-pocket costs for assessments, treatment, rehabilitation, transport, and Assistive technology are often unaffordable.
- o Families with disabilities typically have lower income, making high medical and rehabilitation costs especially burdensome.

Solution: Addressing Financial and Economic Barriers

- o Expand health insurance coverage (e.g., SHA - SHIF schemes) to fully include rehabilitation and Assistive technology.
- o Provide targeted subsidies or vouchers for essential Assistive technology and rehabilitation services.
- o Partner with NGOs, faith-based organizations, and private sector to establish assistive device banks at county level.
- o Integrate rehabilitation and assistive technology services into primary health care, reducing travel and costs.
- o Support community-based rehabilitation (CBR) programs that deliver services closer to where people live.

6. Policy and System Barriers

- o Weak implementation of disability-inclusive health policies.
- o Lack of disability-disaggregated data in health systems, leading to poor planning.
- o Some policies may be discriminatory or create unintentional barriers.

Solutions: Addressing Policy and System Barriers

- o Strengthen enforcement of disability-inclusive health policies at national and county levels.
- o Collect and use disability-disaggregated data in health information systems for better planning and budgeting.
- o Ensure all new health facilities and renovations follow universal design standards for accessibility.
- o Include representation of persons with disabilities in health policy-making and advisory boards.
- o Develop standard operating procedures (SOPs) for disability-inclusive service delivery in all levels of care.
- o Allocate dedicated budget lines for disability inclusion in health programs at both national and county levels.

TOPIC 2.2: HEALTH PROMOTION

Module outcome

By the end of this session, the learner should be able to:

- o Define the concept of health promotion.
- o Describe key concepts of health promotion.
- o Identify reasons for carrying out health promotion.
- o Explain why health promotion is important for individuals and communities.
- o List common health promotion activities

2.2.1 The Concept of Health Promotion

Methodology:	Interactive Lecture + Brainstorming.
Time:	25 minutes.
Facilitator Instructions	
1. Ask learners: "When you hear the word 'health promotion,' what comes to mind?" (5 min). 2. Present the WHO definition and explain in simple terms (10 min). 3. Brainstorm examples of health promotion in daily life (e.g., handwashing campaigns, anti-smoking ads) (10 min).	

Definition (WHO): Health promotion is the process of using education and strategy to help people and communities voluntarily adopt behaviors that improve their overall health and prevent disease.

It goes beyond treating illness, it focuses on prevention and creating supportive environments for well-being.

Health promotion involves:

1. Educating people.
2. Influencing behavior.
3. Creating policies and environments that support healthy living

Health promotion focuses on addressing those determinants of health that can potentially be modified, such as individual health behaviors and lifestyles, income and social status, education, employment and working conditions, access to appropriate health services and the physical environment.

Health promotion aims to increase control over health and its determinants. The health potential of persons with disabilities is frequently overlooked and as a result they are often excluded from health promotion activities.

Take away: Health promotion is a proactive approach that uses education and strategies to empower individuals and communities to improve their own well-being and prevent illness.

2.2.2: Key Concepts of Health Promotion

Methodology:	Small group work + plenary discussion.
Time:	30 minutes.
Facilitator Instruction;	
1. Divide learners into groups; assign each a concept (empowerment, participation, equity, holistic approach, multi-sectoral action) (10 min). 2. Each group discusses and writes examples of how their concept looks in real life (10 min). 3. Groups present briefly; facilitator links back to definitions (10 min).	

Key concepts in Health promotion

Health promotion for persons with disabilities

Many persons with disabilities have as much need for health promotion as does the general population, if not more. Persons with disabilities are at risk of the same health conditions as people in the general population but they may also have additional health problems due to greater susceptibility to health conditions. Often, persons with disabilities and their family members have very little awareness of how to achieve or maintain good health.

Barriers to health promotion

Persons with disabilities often experience poorer health outcomes compared to the general population due to numerous barriers encountered when attempting to improve their health. Addressing these barriers will facilitate greater participation of persons with disabilities in health promotion activities.

Support health promotion campaigns

Health promotion campaigns can positively influence the health of individuals, communities and broader populations by informing, encouraging and motivating behavior change. This can be achieved by showing positive images of persons with disabilities, utilizing accessible formats for persons with disabilities, identifying existing resources within the community and encouraging increased coverage of disability-related health issues. Supporting the development of local health promotion campaigns to address disability-related issues not covered by existing campaigns.

Strengthen personal knowledge and skills

Health information and education empower persons with disabilities and their families to build the necessary knowledge and life skills for maintaining and improving their health. They can learn about disease risk factors, good hygiene, healthy eating choices, the importance of physical activity and other protective factors through structured sessions.

Link people to self-help groups

Self-help groups enable people to come together in small numbers to share common experiences, situations or problems with each other. For many people the opportunity to receive support and practical advice from someone else who has a similar problem is more useful than receiving advice from a health worker.

Educate health-care providers

Health-care providers are a trusted source of health-related information and have the potential to positively influence the health of others. There is a need to work with these providers to ensure they have adequate knowledge about disability and include persons with disabilities in all their health promotion activities.

Create supportive environments

CBID Programmes can work with community health centers, hospitals, schools, worksites and recreational facilities and with key stakeholders to create supportive physical and social environments for people with disabilities, as well as to enable them to achieve optimal health.

Take away:

1. Empowerment: Enabling people to take control of their health.
2. Participation: Communities should be actively involved in decisions.
3. Equity: Everyone should have fair access to health resources.
4. Holistic approach: Health promotion addresses physical, mental, and social well-being.
5. Multi-sectoral action: Health promotion involves schools, communities, government, and workplaces.

2.2.3 Identify Reasons for Carrying Out Health Promotion

Methodology:	Case study + discussion.
Time:	25 minutes.
Facilitator Instructions;	
1. Present a short case: “A community with rising cases of diabetes due to poor diet.” (5 min). 2. Ask learners: “What could health promotion do to help this community?” (10 min). 3. Summarize the reasons and emphasize prevention and empowerment (10 min).	

Facilitator notes: Ask the participants why they should do health promotion

- o To ensure persons with disabilities and their families are reached by the same health promotion messages as are members of the general community.
- o To ensure health promotion materials and Programmes are designed or adapted to meet the specific needs of persons with disabilities and their families.
- o To impart persons with disabilities and their families with the knowledge, skills and support to assist them to achieve good levels of health.
- o To ensure health-care personnel have improved awareness about the general and specific health needs of persons with disabilities and respond to these through relevant health promotion activities.
- o To sensitize the community to provide a supportive environment for persons with disabilities to participate in activities which promote their health.
- o To ensure access and inclusion for persons with disabilities and their family members in health promotion activities.
- o To ensure that persons with disabilities and their families know the importance of maintaining good health and encourage them to actively participate in health promoting activities.

Take away:

1. Preventing diseases before they occur.
2. Reducing healthcare costs.
3. Improving quality of life.
4. Encouraging healthier behaviors (e.g., good nutrition, exercise).
5. Supporting national and global health goals.

2.2.4 Explain Why Health Promotion is Important for Individuals and Communities

Methodology:	Group discussion + role play.
Time:	35 minutes.
Facilitator Instructions	
1. Ask learners to discuss in pairs: “Why is health promotion important to me? Why is it important to my community?” (10 min). 2. Invite 2–3 groups to role play (e.g., a health worker convincing community leader about the importance of health promotion) (15 min). 3. Summarize key points and link to public health goals (10 min).	

Content

- o For individuals: Promotes healthy lifestyles, prevents illness, improves productivity and longevity.
- o For communities: Reduces disease burden, creates healthier environments, lowers health system costs, strengthens social cohesion.
- o Example: Immunization protects individuals and prevents outbreaks in communities.

Take away: Investing in health promotion benefits everyone by preventing illness in individuals and creating stronger, healthier communities.

2.2.5 Common Health Promotion Activities

Methodology:	Brainstorming + poster/chart exercise.
Time:	30 minutes.
Facilitator Instructions	
1. Ask learners to brainstorm activities they have seen in their community (10 min). 2. On flip chart, group responses into categories (education, prevention, policy, community action) (10 min). 3. Add missing examples and highlight diverse approaches (10 min).	

Health Promotion Activities

- o **Community Awareness Campaigns:** Organize inclusive health talks, radio programs, and community dialogues on healthy lifestyles, disease prevention, disability rights, and mental well-being.
- o **Accessible Health Education Materials:** Develop and distribute health information in multiple formats (braille, large print, audio, pictorial, and easy-to-read versions) to reach persons with different disabilities.
- o **Peer Support & Role Models:** Engage persons with disabilities as peer educators and health champions to promote positive health-seeking behavior within their communities.
- o **Inclusive Screening & Check-ups:** Conduct community-based medical screenings (blood pressure, HIV, diabetes, maternal and child health) with mobile or outreach clinics that are physically and communication accessible.
- o **Nutrition & Wellness Programs:** Promote healthy diets through community kitchens, accessible cooking demonstrations, and home-based gardening initiatives.
- o **Physical Activity & Rehabilitation:** Organize inclusive sports, exercise groups, and physiotherapy sessions suitable for persons with varying abilities.
- o **Training for Health Workers:** Build capacity of health personnel on disability inclusion, respectful communication, and accessible service provision.
- o **Mental Health & Psychosocial Support:** Provide counseling, peer support groups, and safe spaces for emotional well-being, especially for persons with psychosocial disabilities.

- o **Community-Led Advocacy:** Mobilize community groups to advocate for improved accessibility in health facilities, transport, and services.
- o **School & Youth Health Clubs:** Create inclusive platforms for children and young people with and without disabilities to learn and practice healthy behaviors together.

Take away:

1. Health education campaigns (nutrition, hygiene, safe sex).
2. Immunization programs.
3. Screening and early detection (blood pressure, cancer).
4. Policy initiatives (tobacco control, road safety laws).
5. Community mobilization (clean-up campaigns, physical activity programs).
6. Workplace wellness programs.

SHORT STORY ON HEALTH PROMOTION

In the village of Kazi, the sun always set like a proud king, painting the sky with fiery colors. But for Mama Kamau, who sat on her small porch, the world felt a little smaller. Her knees, stiff with age, kept her from the village center where the health worker, Wafula held his weekly talks. She would watch the other women, their baskets balanced on their heads, walk past, and feel a pang of loneliness. She was a storyteller, a giver of advice, but her world was now just her small compound.

One day, Wafula noticed her. Instead of waiting for her to come to him, he brought his lesson to her. He sat on a low stool in her dusty yard, unrolling a large chart with colorful pictures of local vegetables: green *sukuma wiki*, sweet potatoes, and bright red tomatoes. He explained how these foods could

keep bodies strong and minds sharp. The children, curious, gathered around, giggling as he drew more pictures.

Mama Kamau, a woman of deep wisdom, listened intently. She shared a secret of her own: a special herb her grandmother used to soothe swollen joints. Soon, her porch became the new community health hub. People started coming not just for Wafula's lessons, but to hear Mama Kamau's stories. She became a health champion, her gentle voice and lived experience a powerful message.

The take-away was simple: **a community that includes everyone, regardless of their ability, becomes stronger and healthier for it.** Mama Kamau, once isolated, now taught them that true health is a shared journey, walked together.

TOPIC 2.3: PREVENTION

Module outcomes:

By the end of the session, the learner should be able to: -

- Define the concept of prevention of health conditions
- Identify the different levels of prevention of health conditions
- Describe common activities under each level of prevention

2.3.1 Definition of Prevention of Health Conditions

Methodology:	Interactive lecture + brainstorming.
Time:	15 minutes.
Facilitator Instructions	
1. Ask learners: "What does prevention mean to you in everyday life?" (3 min). 2. Present the health-related definition of prevention (5 min). 3. Brainstorm quick examples of prevention activities learners know (7 min).	

Definition: Prevention in health refers to actions taken to avoid the occurrence of disease, reduce its impact, or delay its progression. It focuses on maintaining well-being and reducing risk factors before health problems become severe.

Prevention is very closely linked with health promotion. Prevention of health conditions (e.g. diseases, disorders, injuries) involves primary prevention (avoidance), secondary prevention (early detection and early treatment) and tertiary prevention (rehabilitation) measures. The focus of this element is mainly on primary prevention.

Example: Immunization prevents infectious diseases such as polio or measles.

Take away; Prevention in health means taking action to stop diseases from happening or getting worse, with a primary focus on avoiding them in the first place.

2.3.2 Levels of Prevention of Health Conditions

Methodology:	Mini-lecture + group discussion with examples.
Time:	25 minutes.
Facilitator Instructions	
1. Present the three levels of prevention using a simple diagram (pyramid or timeline) (10 min). 2. Divide learners into three groups; assign each group one level of prevention. Ask them to come up with two examples from their community (10 min). 3. Each group shares in plenary; facilitator clarifies and links examples to the correct level (5 min).	

Levels of prevention

Prevention interventions can be at one of three levels.

1. Primary prevention

Primary prevention is directed at avoidance and uses interventions that prevent health conditions from occurring. These interventions are mainly aimed at people (e.g. changing health behaviors, immunization, nutrition) and the environments in which they live (safe water supplies, sanitation, good living and working conditions).

2. Secondary prevention

This is the early detection and early treatment of health conditions, with the aim of curing or lessening their impacts. Examples of early detection include mammograms to detect breast cancer and eye examinations to detect cataracts; examples of early treatment include treatment of trachoma with antibiotics to prevent blindness, multidrug treatment of leprosy to prevent disease progression and appropriate handling of a

fractured bone to promote proper healing and prevention of deformity.

3. Tertiary prevention

This aims to limit or reverse the impact of already existing health conditions and impairments; it includes rehabilitation services and interventions that aim to prevent activity limitations and to promote independence, participation and inclusion.

The 3 main levels of prevention include:

1. Primary prevention : Actions to prevent a disease before it occurs.

- Example: Vaccination, health education, safe water, good nutrition.

2. Secondary prevention : Early detection and treatment to stop disease from progressing.

- Example: Screening for high blood pressure, cancer screening, HIV testing.

3. Tertiary prevention : Reducing complications and improving quality of life after disease has developed.

- Example: Rehabilitation after a stroke, provision of Assistive technology, support groups.

Take away; Prevention has three main levels: **primary**, which stops a problem from starting; **secondary**, which detects and treats it early; and **tertiary**, which manages an existing condition to limit its impact.

2.3.3 Activities under each level of prevention

Methodology:	Group work + chart/poster presentation+ role play
Time:	30 minutes.
Facilitator Instructions	
1. Divide participants into 3 groups; assign each group one level of prevention. (5 min). 2. Ask them to list activities under their assigned level and prepare a simple chart or poster (10 min). 3. Groups present their findings (10 min). 4. The facilitator summarizes using a master chart (pre-prepared) showing activities under all levels (5 min).	

Activities on prevention of health conditions

- o Facilitate access to existing prevention Programmes: Gather information about existing prevention activities in their communities and work with prevention Programmes to include people with disabilities, thus ensuring greater coverage.
- o Promote healthy behaviors and lifestyles. Healthy behaviors, such as not smoking, drinking only small amounts of alcohol, healthy eating, exercising regularly and wearing condoms during sex, can reduce the risk of developing health problems. Prevention programs often utilize health promotion strategies to encourage healthy behaviors, such as awareness campaigns to communicate prevention messages within communities and provide education for individuals.
- o Encourage immunization: Within each community, immunization Programmes should be available for specific diseases and for high-risk groups, e.g. poliomyelitis, diphtheria, tetanus and measles vaccinations for infants and young children and tetanus vaccination for pregnant women.
- o Ensure proper nutrition. Poor nutrition (malnutrition) usually results from not getting the right amount of nutrients in a diet. It can involve either undernutrition (not getting enough nutrients)

or over nutrition (getting too much nutrients, particularly calories) and poor eating habits and is a common cause of health problems. Ensuring adequate food and nutrition in communities is the responsibility of many development sectors.

- o Facilitate access to maternal and child health care. Antenatal care, skilled care during delivery and postnatal care reduce the risk of mothers and babies developing health conditions and/or impairments that may lead to disability.
- o Promote clean water and sanitation: Water and sanitation measures contribute to improving healthy living and minimizing disability
- o Help to prevent injuries. Many disabilities are caused by accidents at home, at work or in the community. Often adults and children with disabilities are also at higher risk of injury.
- o Help to prevent secondary conditions. People of all ages with disabilities are at risk of secondary conditions. CBID Programmes can promote primary prevention strategies to reduce the likelihood that people with disabilities will develop these conditions.

Examples of common activities:

1. Primary prevention activities:

- Immunization campaigns
- Health education on hygiene, nutrition, lifestyle
- Safe water and sanitation projects
- Road safety laws (helmets, seatbelts)

2. Secondary prevention activities:

- Screening for diabetes, hypertension, cancers
- Growth monitoring for children
- Early diagnosis and prompt treatment (e.g., malaria RDTs and treatment)

3. Tertiary prevention activities:

- Physiotherapy and rehabilitation
- Assistive technology (wheelchairs, hearing aids)
- Long-term care and counseling Community-based rehabilitation services

Take away:

- o **Prevention is multifaceted:** It involves a wide range of activities, from immunization and nutrition to promoting healthy behaviors and ensuring access to clean water.
- o **Accessibility is crucial:** It's important to ensure that people with disabilities are included in all prevention programs and have access to the same resources as the general population.
- o **Prevention works at all levels:** From individual actions like wearing a seatbelt, to community-wide efforts like sanitation projects, to managing existing conditions with rehabilitation, prevention is a continuous process.

SHORT ROLE PLAY ON PREVENTION OF HEALTH CONDITIONS

Setting:	A small, busy community health clinic in a village in Kenya.
Characters:	• Mama Mwangi: A respected elder in her late 50s.
	• Juma: A young health worker in his 20s.

(The scene opens with Mama Mwangi sitting patiently outside the clinic. Juma, the health worker, walks over to greet her.)

Juma: *(Smiling warmly)* Jambo, Mama Mwangi. It's good to see you. How are you today?

Mama Mwangi: *Jambo, kijana.* I came for malaria medicine for my grandson, but you have me thinking. You spoke of three kinds of prevention. What is the difference again?

Juma: *(Pulls up a stool to sit with her)* It's simple. Look at your home.

Primary prevention is like fixing a hole in your roof before the rain comes. We do this with things like building proper latrines, making sure water is clean, and giving children their immunizations to stop sickness before it starts.

Mama Mwangi: And what about the other two?

Juma: Secondary prevention is like patching a small leak in your roof as soon as you see a drip. We do this by checking for diseases early, like when we screen for high blood pressure or diabetes to catch it before it becomes severe.

Tertiary prevention is for after the big storm has hit and a wall has fallen. It's about rebuilding and strengthening. Bwana Otieno, who had a stroke, is a good example. His physiotherapy is to help him walk again and prevent more damage. So, one stops it from happening, one catches it early, and one helps manage it after.

Total Time Allocation

Session 1:	Definition	15 min
Session 2:	Levels	25 min
Session 3:	Activities	30 min
Total = 70 minutes (1 hour 10 minutes)		

TOPIC 2.4: MEDICAL CARE

Module outcome

By the end of the session, the learner should be able to: -

- o Define the concept of medical care.
- o Identify the different types of medical care.
- o Describe common activities under medical care.

2.4.1 Definition of Medical Care

Methodology:	Interactive lecture + brainstorming.
Time:	15 minutes.
Facilitator Instructions	
1. Begin with a question: "When you hear 'medical care,' what comes to mind?" (3 min). 2. Present the formal definition and simplify it for learners (7 min). 3. Ask participants for quick examples of when they or their families have received medical care (5 min).	

Definition: Medical care refers to services provided by trained health professionals to diagnose, treat, and prevent illnesses or injuries, and to promote overall health.

Medical care refers to the early identification, assessment and treatment of health conditions and their resulting impairments, with the aim of curing or limiting their impacts on individuals. Medical care can take place at the primary, secondary or tertiary level of the health-care system.

It involves both curative and preventive aspects, delivered in clinics, hospitals, community health centers, and even at home.

Example: Visiting a doctor for treatment of malaria or receiving antibiotics for an infection.

2.4.2 Types of Medical Care

Methodology:	Mini-lecture + group work
Time:	25 minutes.
Facilitator Instructions	
1. Present the five types of medical care with examples (10 min). 2. Divide participants into small groups; assign each group one type of medical care. Ask them to discuss where they see it in their community (10 min). 3. Each group shares one example in plenary (5 min).	

Types of medical care

Many health systems in low-income countries have three levels of health care: primary, secondary and tertiary. These are usually linked to each other by a referral system; e.g. Primary health care workers refer people to secondary care when needed. While there is often overlap between each level, e.g. primary health care might be provided in a place that normally provides secondary health care, it is important to understand the basic differences between the levels so they can facilitate access for people with disabilities and their family members.

- 1. Primary level** of care refers to basic health care at the community level. It is usually provided through health centers or clinics and is usually the first contact people have with the health system. Medical care provided at primary level includes short simple treatments for acute conditions (e.g. infections) and routine management of chronic conditions (e.g. leprosy, epilepsy, tuberculosis, diabetes).
- 2. Secondary level** of care refers to more specialized medical services that are provided by large clinics or hospitals which are usually present at the district level. Primary health care provides an important link to secondary care through referral mechanisms.
- 3. Tertiary level** of care is highly specialized medical care. It is provided by specialized medical professionals in association with nurses and paramedical staff and involves the use of specialized technology. Medical care provided at the tertiary level might include brain surgery, cancer care or orthopedic surgery.

In Summary, Medical care can be grouped into several types:

1. Primary medical care

- o First point of contact with the health system.
- o Example: Outpatient clinics, dispensaries, health centers.

2. Secondary medical care

- o Specialized care, usually in hospitals, upon referral.
- o Example: Treatment in a district or county hospital.

3. Tertiary medical care

- o Highly specialized care in referral hospitals.
- o Example: Surgery, intensive care, cancer treatment.

4. Emergency care

- o Immediate treatment for life-threatening conditions.
- o Example: First aid, ambulance services, emergency room care.

5. Palliative/rehabilitative care

- o Care that focuses on comfort, long-term recovery, or improving quality of life.
- o Example: Pain management for terminal illness, physiotherapy, occupational therapy.

Take away; Primary, secondary, and tertiary care are the three levels of medical care, with each level offering a different degree of specialization and treatment. Primary care is the first point of contact, while secondary and tertiary care provide increasingly specialized services, often through a referral system.

2.4.3 Common Activities Under Medical Care

Methodology:	Group activity + plenary discussion.
Time:	30 minutes.
Facilitator Instructions	
1. Write the five types of medical care on a flip chart. 2. Ask participants to brainstorm and list activities that fall under each type (10 min). 3. Groups present back; facilitator organizes responses under the correct category (15 min). 4. Summarize with a chart of “Types of medical care → Activities” (5 min).	

Activities on medical care

Facilitators notes: *Trainer asks participants to brainstorm on the activities on medical care*

- **Gather information about medical services;** Knowledge of the medical services available at primary, secondary and tertiary levels of the health system is essential for assisting persons with disabilities and their families to access medical care and support
- **Assist with early identification;** establish a mechanism for the early identification of health conditions and impairments associated with disability in partnership with primary health care personnel.
- **Ensure access to early treatment:** promote and encourage collaboration between persons with disabilities, their families and primary health care workers to increase access to medical care services at all levels.
- **Facilitate access to surgical care;** Some persons with disabilities may require surgical care. When combined with follow-up care and rehabilitation, surgery can correct impairments, prevent them from becoming worse and contribute to improved functioning

- **Promote self-management of chronic conditions:** Assist persons with disabilities and their families to become aware of their right to medical care and to learn skills that enable them to manage their chronic health conditions.
- **Build relationships with medical care providers;** Medical personnel often have limited knowledge about disability and how best to enable access for persons with disabilities to medical care services.

Examples of activities include:

- o **Diagnosis and treatment** – laboratory tests, prescribing medicines.
- o **Preventive services** – immunization, antenatal care, health check-ups.
- o **Emergency interventions** – wound care, resuscitation, surgery.
- o **Rehabilitation** – physiotherapy, occupational therapy, provision of Assistive technology.
- o **Palliative/supportive care** – pain relief, counseling, home-based care.
- o **Health education** – advising patients on nutrition, hygiene, or medication use.

Total Time Allocation		
Session 1:	Definition	15 min
Session 2:	Types	25 min
Session 3:	Activities	30 min
Total = 70 minutes (1 hour 10 minutes)		

TOPIC 2.5: REHABILITATION AND HABILITATION

Module outcome

By the end of the session, the learner should be able to:

- Define the concept of rehabilitation.
- Define the concept of habilitation.
- Identify activities under rehabilitation and habilitation.
- Differentiate the main types of rehabilitation services.
- Describe the roles of rehabilitation service providers.

2.5.1 Definition of Rehabilitation

Methodology:	Interactive lecture + Q&A.
Time:	10 minutes.
Facilitator Instructions	
1. Ask: “What comes to your mind when you hear the word ‘rehabilitation’?” (3 min). 2. Present the WHO definition and explain using simple examples (5 min). 3. Allow 1–2 learners to give real-life examples from their community (2 min).	

Rehabilitation: A set of measures that help people with health conditions achieve and maintain optimal functioning in interaction with their environment.

Rehabilitation is a series of interventions designed to help persons with disabilities attain and sustain optimal functioning in their daily environments. It applies to both those who acquire disabilities later in

life and those who are born with them. Rehabilitation services vary from basic support to specialized care and can be provided in diverse settings, including hospitals, homes, and community spaces. Although often initiated within the health sector, successful rehabilitation requires collaboration across multiple sectors.

Focuses on restoring skills or abilities that were lost due to illness, injury, or disability. Examples: Physiotherapy after a fracture, speech therapy after a stroke, or counseling after trauma.

2.5.2 Definition of Habilitation

Methodology:	Mini-lecture + comparison exercise.
Time:	10 minutes.
Facilitator Instructions	
1. Write the two terms on the board: “Rehabilitation” vs. “Habilitation.” 2. Give the definitions and simple examples of each (5 min). 3. Ask learners to pair up and come up with one example of rehabilitation and one example of habilitation (5 min).	

Habilitation: Services and support that help people, especially children with disabilities, develop new skills and abilities they never had before.

Habilitation supports individuals who acquire disabilities at birth or early childhood and have not yet learned to function without them. In contrast, rehabilitation focuses on helping those who have lost certain abilities due to illness or injury, enabling them to relearn essential skills and regain maximum independence in daily activities.

Different from rehabilitation because it is about *learning* rather than *restoring*.

Examples: Teaching a child with cerebral palsy how to walk using mobility aids; training a child with hearing loss to use hearing aids or sign language.

2.5.3 Activities under Rehabilitation and Habilitation

Methodology:	Group work + poster/flipchart presentation.
Time:	20 minutes.
Facilitator Instructions	
1. Divide learners into 2 groups: one for rehabilitation activities and the other for habilitation activities (5 min). 2. Ask them to list examples from their community on flip charts (10 min). 3. Each group presents; the facilitator clarifies and adds missing examples (5 min).	

Rehabilitation activities:

- Physical therapy after injury
- Speech therapy after stroke
- Counseling for trauma survivors
- Assistive technology provision (wheelchairs, prosthetics)

Habilitation activities:

- Early intervention programs for children with developmental delays
- Teaching use of Assistive technology (e.g., hearing aids, communication boards)
- Occupational therapy for independent living skills
- Special education support

2.5.4 Types of Rehabilitation Services

Methodology:	Interactive lecture + case examples.
Time:	20 minutes.
Facilitator Instructions:	
1. Present each type with a short example (10 min). 2. Share a short case: e.g., “A person with spinal cord injury, what types of rehabilitation would they need?” and invite responses (10 min).	

Content

1. Restoring Functionality

- Help persons with temporary or permanent impairments regain independence in daily living activities.
- Provide physical, occupational, and speech therapy to improve mobility, communication, and self-care skills.

2. Enhancing Participation

- Support persons with disabilities to engage fully in education, employment, family, and community life.
- Adapt environments, tools, and practices to promote inclusion.

3. Preventing Further Complications

- Provide early interventions to reduce the risk of secondary health conditions (e.g., pressure sores, contractures, depression).
- Educate individuals and caregivers on self-care and safe practices.

4. Capacity Building

- Train caregivers, families, and community health workers to support rehabilitation at home and in community settings.
- Develop peer-support networks for sharing experiences and solutions.

5. Facilitating Access to Assistive Technology

- Provide, maintain, and train users on Assistive technology such as wheelchairs, crutches, hearing aids, prosthetics, and communication boards.
- Advocate for affordable and locally available assistive products.

6. Promoting Social Inclusion and Rights

- Reduce stigma and discrimination by raising awareness about disability rights and abilities.
- Advocate for inclusive policies in education, employment, and health.

7. Supporting Mental Health and Psychosocial Well-being

- Offer counseling, psychosocial support, and safe spaces for persons with disabilities and their families.
- Promote resilience and coping strategies.

8. Community-Based Rehabilitation (CBR/CBID) Integration

- Work hand-in-hand with communities to design locally relevant rehabilitation services.
- Link persons with disabilities to community resources, self-help groups, and livelihood opportunities.

Facilitate rehabilitation activities

Facilitate home and/or community-based therapy services and provide assistance to persons with a wide range of impairments, enabling them to maintain and maximize their function within their home and community

Develop and distribute resource materials

Disability booklets and manuals can be useful tools for rehabilitation. These resources can be used to guide rehabilitation efforts, particularly in areas where access to rehabilitation professionals is limited. These resources may also provide valuable information for the wider community as well as the many different services and sectors involved in rehabilitation activities.

Provide training

CBID personnel need training to ensure they are able to facilitate access to rehabilitation services and provide appropriate services at community level. Many organizations have developed suitable training programs. CBID personnel require a good understanding of the role of rehabilitation personnel, e.g. physiotherapists, occupational therapists, speech therapists, audiologists, mobility trainers, prosthetics/orthotists, medical and paramedical personnel and of how they can be of benefit to people with different impairments.

1. **Physical rehabilitation** – physiotherapy, occupational therapy, prosthetics & orthotics.
2. **Speech and language therapy** – communication and swallowing support.
3. **Psychosocial rehabilitation** – counseling, mental health support, community reintegration.
4. **Vocational rehabilitation** – job training, skill-building for employment.
5. **Community-based rehabilitation (CBR)** – services delivered at community level integrating health, education, livelihood, and social inclusion.

2.5.5 Roles of Rehabilitation Service Providers

Methodology:	Case Scenario/ Role Play + discussion.
Time:	25 minutes.
Facilitator Instructions	
1. Assign learners' roles (therapist, doctor, caregiver, patient) and give them a simple scenario (e.g., stroke patient coming for rehab) (10 min). 2. Role play how different providers would support the patient (10 min). 3. Facilitate discussion and summarize the roles (5 min).	

Trainer Notes: Trainer to prompt trainees to discuss the roles of rehabilitative service providers.

1. Physiotherapists

Roles:

- Use exercises, massage, and physical treatments.
- Reduce pain and stiffness.
- Improve balance and coordination.
- Help people walk or use their bodies again.

2. Occupational Therapists

Roles:

- Teach skills for daily life (eating, bathing, dressing, working, schooling)
- Recommend and train people to use special tools or equipment.
- Support people to return to school, work, or community life.

3. Speech Therapists (Speech and Language Therapists)

Roles:

Improve speech and language skills.

- Teach alternative communication methods (sign language, picture boards).
- Support safe eating and swallowing.

4. Orthopedic Specialists

Roles:

- Diagnose and treat bone and joint problems (fractures, arthritis, back pain).
- Perform surgeries when needed.
- Work with physiotherapists to support recovery.

5. Audiologists

Roles:

- Test hearing and identify hearing loss.
- Provide and fit hearing aids.
- Teach ways to improve communication.
- Support people with hearing loss to participate fully in daily life.

Types of rehabilitation services and their definition.

1. **Physiotherapists:** Health professionals who help people move better and become stronger after illness, injury, or disability.
2. **Occupational Therapists:** Health professionals who help people take care of themselves and live more independently.
3. **Speech Therapists (Speech and Language Therapists):** Health professionals who help people who have trouble speaking, understanding, or swallowing.
4. **Orthopedic Specialists:** Medical doctors who treat problems of bones, joints, and muscles.
5. **Audiologists:** Health professionals who deal with hearing and balance problems.
6. **Physiotherapists:** Improve mobility, strength, and physical function.
7. **Occupational therapists:** Help people regain independence in daily activities.
8. **Speech and language therapists:** Support communication and swallowing.
9. **Prosthetics & orthoptists:** Design and fit Assistive technology.
10. **Psychologists/counselors:** Provide mental health and emotional support.
11. **Medical doctors/nurses:** Provide medical oversight and treatment.

12. Community health workers & caregivers: Offer daily support and follow-up at home.

Session 5: The trainer asks the trainees to discuss a case story and provide answers

Case scenario

Client: John, a 12-year-old boy	What happened: John was in a road accident and broke his right leg. The leg healed, but it became weak, and now he has trouble walking. His right hand also became stiff, so he finds it hard to eat, dress, or write.
---------------------------------	--

Challenges John faces:

1. He limps when walking because his leg is weak.
2. His right hand is stiff, making daily tasks difficult.
3. He needs a special support (brace) to walk safely without falling.
4. He struggles to care for himself and to do schoolwork on his own.

How Rehabilitation Can Help

1. Physiotherapy (movement therapy)
2. Make his leg stronger.
3. Help him walk better and balance.
4. Reduce stiffness with exercises and stretches.
5. Occupational Therapy (daily life skills):
6. Teach him how to use his hand for eating, dressing, and writing.
7. Show easier ways to do school and home activities.
8. Suggest simple tools like special spoons or writing aids.
9. Orthotist Services (braces and supports):
10. Provide a leg brace to support his weak muscles.
11. Help him learn how to use the brace for safe walking.

Total Time Allocation		
Session 1:	Definition of rehabilitation	10 min
Session 2:	Definition of habilitation	10 min
Session 3:	Activities	20 min
Session 4:	Types	20 min
Session 5:	Roles	25 min
Total = 85 minutes (~1 hour 25 minutes)		

TOPIC 2.6. ASSISTIVE TECHNOLOGY

Module outcome:

By the end of the session, the learner should be able to:

- o Define the concept of assistive technology.
- o Identify the different types of assistive technology.

Methodology:	Interactive lecture + brainstorming
Time:	15 minutes.
Facilitator Instructions	
1. Begin by asking learners: “What comes to your mind when you hear ‘assistive device’?” (3 min). 2. Present the formal definition with 2–3 simple examples (5 min). 3. Ask learners to brainstorm examples they have seen in their communities (7 min).	

2.6.1 Definition of Assistive technology

Definition (WHO)

Assistive technology : Are tools, equipment, systems, or products that help people with disabilities or health conditions improve their functioning and independence.

Assistive devices: Assistive devices are defined as any external product (including software) whose primary purpose is to maintain or improve an individual’s functioning and independence, thereby promoting their well-being. (WHO definition).

They support people in carrying out daily activities, improving mobility, communication, vision, hearing, and self-care.

- Assistive technology are external devices that are designed, made, or adapted to assist a person to perform a particular task. The majority of persons with disabilities depend on Assistive technology to enable them to carry out daily activities and participate actively and productively in community life.
- Access to assistive technology is essential for many persons with disabilities and is an important part of any development strategy. Without assistive technology, persons with disabilities may never be educated or able to work, so the cycle of poverty continues.

Examples: Wheelchairs, crutches, hearing aids, spectacles, prosthetics, walking sticks, magnifiers. Assistive technology help promote independence, dignity, and participation in society.

2.6.2 Types of Assistive technology

Methodology:	Group activity + object demonstration (where available).
Time:	30 minutes.
Facilitator Instructions	
1. Present the main categories of Assistive technology using a chart or slides (10 min). 2. Divide learners into groups and assign each group one category. Ask them to list examples and explain how they are used (10 min). 3. Each group presents in plenary; the facilitator summarizes and adds missing examples (10 min).	
If available, show real devices or pictures to make it more practical.	

Types of Assistive technology

Mobility devices

Mobility devices assist people to walk or move and may include:

- **Wheelchairs**



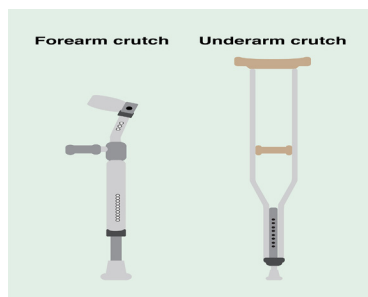
(Provide a voice description for the visually impaired)

- **Tricycles**



(Provide a voice description for the visually impaired)

- **Crutches**



(Provide a voice description for the visually impaired)

- **Walking sticks/canes**



(Provide a voice description for the visually impaired)

- **Walking frames/walkers.**



(Provide a voice description for the visually impaired)

Prosthetics, orthotics and orthopedic shoes

These are usually custom-made devices which replace, support or correct body parts. They are designed, manufactured and fitted in specialized workshops or centers by trained prosthetic/orthotics personnel and include:

- **Prostheses, e.g. Artificial legs or hands**

(Provide a voice description for the visually impaired)

- **Orthoses, e.g., Spinal braces, hand/leg splints or calipers**



(Provide a voice description for the visually impaired)

- **Orthopedic shoes.**



(Provide a voice description for the visually impaired)

Daily living devices

These devices enable persons with disabilities to complete the activities of daily living (e.g. eating, bathing, dressing, toileting, home maintenance). There are many examples of these devices, including:

- **Adapted cutlery and cups**



(Provide a voice description for the visually impaired)



(Provide a voice description for the visually impaired)

- **Shower seats and stools**



(Provide a voice description for the visually impaired)

- **Toilet seats and frames**



(Provide a voice description for the visually impaired)

- **Dressing sticks**



Vision devices

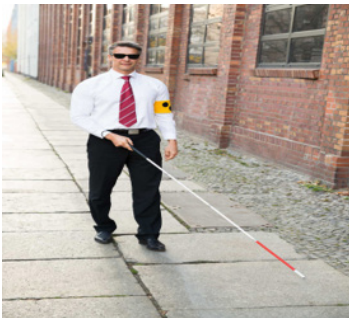
Low vision or blindness has a great impact on a person's ability to carry out important life activities.

- **Large print books**
- **Magnifiers**



(Provide a voice description for the visually impaired)

- **Eyeglasses/spectacles**
- **White canes**



(Provide a voice description for the visually impaired)

- **Braille systems for reading and writing**
- **Audio devices, e.g. Radios, talking books, mobile phones**
- **Screen readers for computers, e.g. Jaws (job access with Speech)**

Hearing devices

Hearing loss affects a person's ability to communicate and interact with others;
Devices include:

- **Hearing aids.**



(provide a voice description for the visually impaired)

- **Headphones for listening to the television**
- **Amplified telephones**
- **TTY/TTD (telecommunication devices)**
- **Visual systems to provide cues, e.g. A light when the doorbell is ringing**

Communication devices

- Augmentative and alternative communication devices help individuals who struggle with speech comprehension and production. These tools either enhance spoken communication (augmentative) or serve as substitutes for speech (alternative). Examples include communication boards featuring pictures, symbols, or letters.
- Request cards
- Electronic speech output devices
- Computers with specialized equipment and Programmes.

Cognitive devices

Cognition is the ability to understand and process information.

The following devices can assist individuals to remember important tasks/events, manage their time and prepare for activities:

- **Lists**
- **Diaries**
- **Calendars**
- **Schedules**
- **Electronic devices, e.g. Mobile phones, pagers, personal organizers**

Take away; Assistive technology is a broad category of tools and devices designed to help people with disabilities live more independently by improving their mobility, communication, and ability to perform daily tasks. These technologies are crucial for breaking the cycle of poverty and enabling full participation in society.

In Summary, Assistive technology can be grouped into categories:

1. **Mobility devices** – wheelchairs, crutches, canes, prosthetic limbs, walkers.
2. **Hearing devices** – hearing aids, cochlear implants.
3. **Vision devices** – spectacles, magnifiers, white canes, braille materials.
4. **Communication devices** – communication boards, speech-generating devices, sign language apps.
5. **Daily living aids** – adapted utensils, dressing aids, toilet aids, grab bars.
6. **Educational/learning aids** – large-print books, specialized software, adapted keyboards.
7. **Environmental adaptations** – ramps, modified furniture, accessible toilets.

Total Time Allocation

Session 1: Definition – 15 minutes

Session 2: Types – 30 minutes

Total = 45 minutes

MODULE 3 : EDUCATION COMPONENT

Module description

This module will cover introduction to education, early childhood care & education, primary education, secondary & higher education, non-formal education and lifelong learning

Module Outcomes:

By the end of the session the learners will be able to: -

- Describe key concepts in education
- Understand the importance of education in CBID
- Identify and address barriers to education for persons with disabilities
- Explain the benefits of education

Module Topics

SN	Topic Names	Duration (Minutes)
3.1	Introduction to education	30
3.2	Early childhood care and education	60
3.3	Primary education	60
3.4	Secondary and higher learning	60
3.5	Non-formal learning	60
3.6	Lifelong learning	60
	Total	330

TOPIC 3.1: INTRODUCTION TO EDUCATION

Methodology:	Flip Charts, Marker pen, LCD, Masking tape
Time:	30 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: "What is your opinion on meaningful participation of persons with disabilities in education in your community?" (5 min). 2. Define education (10 min). 3. Describe key concepts of education in relation to disability (5min) 4. Divide learners into small groups (5–8 members). Ask each group to identify barriers to meaningful participation in education by persons with disabilities and the solutions for each barrier identified (10 min).	

Notes for Facilitator

Step 1: Facilitator introduces the topic and outlines the objectives of the lesson

Step 2: Learners brainstorm on the key concepts in education in relation to disability

Step 3: Learners are divided into groups to identify the barriers to education and the solutions for Persons with Disabilities

Step 4: Learners present and facilitator provides professional input

Step 5: Learners discuss the strategies for implementing Education component in CBID

3.1.1 Key Concepts

Education

Education is much broader than schooling. Schooling is important, but needs to be seen in the context of a lifelong learning process. Education starts at birth in the home, and continues throughout adult life; it includes formal, informal, non-formal, home-based, community and government initiatives

Right to Education

It is the fundamental right of every child and adult to receive education. It ensures that everyone, regardless of their disability, background, gender, or economic status, has access to learning opportunities. It empowers individuals, promotes equality, and supports personal and social development. Ensuring this right helps build a skilled, informed, and fair society.

Inclusive education

It means giving all learners opportunities to learn together in the same environment. It focuses on creating a supportive atmosphere where every learner feels valued and accepted. It helps remove barriers that prevent some learners from accessing quality education. When learners with different abilities and experiences learn side by side, they develop empathy, cooperation, and social skills that are essential for life. Hence inclusive education is not only about placing all learners in one classroom but also ensuring that all learners have the support, understanding, and opportunities they need to succeed.

Integrated education

Integrated education focuses on placing all learners in the same school or classroom (physical location). However, in this system, the learners are often expected to adapt to the existing teaching methods and environment but goes a step further by ensuring that the education system itself adapts to meet the diverse needs of all learners.

Special Education

It is tailored to meet the needs of individual learners. It entails the provision of extra assistance, adapted programmes, learning environments or specialized equipment, materials or methods (e.g. Braille, audio devices, assistive devices, sign language) to support children in accessing education. However, special education does not have an inclusive setting whereby all learners regardless of ability are in the same classroom by providing equal opportunities and necessary support.

Child Development

Child development refers to the process through which a child grows and changes physically, mentally, emotionally, and socially from birth through adolescence. It involves learning skills such as walking, talking, thinking, and interacting with others. It lays the foundation for a child's future learning, behavior, and health. Early experiences both at home and in school greatly influence how a child grows and learns. Supportive environments, proper nutrition, love, and education help children reach their full potential.

Importance of Education in CBID

Education is a key part of CBID because it empowers people with disabilities and marginalized groups to participate fully in society. It provides knowledge and skills, builds confidence, and creates opportunities for employment and self-reliance. Education also promotes social inclusion by breaking barriers and raising

awareness about rights and equality. In CBID, ensuring access to education for everyone strengthens communities and helps create a fair, inclusive, and sustainable society.

3.1.2 Legal Framework

The right to education is enshrined in Article 24 of the UNCRPD, requiring States to ensure inclusive, quality, and free primary and secondary education on an equal basis with others. In Kenya, this commitment is supported by:

- o **The Constitution (Article 53)** – guarantees the right of every child to free and compulsory basic education.
- o **Basic Education Act (2013)** – promotes inclusive education and reasonable accommodation for learners with disabilities.
- o **Sector Policy for Learners and Trainees with Disabilities (2018)** – provides a framework for inclusive learning environments.
- o **SDG 4** – aims to ensure inclusive and equitable quality education for all.

CBID initiatives should promote inclusive schooling, teacher training on inclusive pedagogy, accessible learning materials, and community awareness on the right to education for learners with disabilities.

3.1.3 Barriers to education for Persons with Disabilities

1. **Environmental Barriers**-Lack of wheelchair-accessible buildings, classrooms, or toilets Inaccessible transportation to and from school Poor classroom layout or infrastructure for mobility aids.
2. **Attitudinal Barriers** -Negative attitudes or discrimination from teachers, peers, or society, low expectations about the abilities of persons with disabilities, Stigma and stereotypes that discourage participation.
3. **Communication Barriers** -Lack of sign language interpreters for students with hearing impairment, no learning materials in Braille or audio format for blind students, poor teacher training on how to communicate with students with different needs.
4. **Institutional Barriers**- Weak enforcement of inclusive education policies, teaching methods not adapted to different learning styles or abilities, standardized exams and assessments that do not accommodate people with disabilities, lack of inclusive learning materials or assistive technologies and insufficient funding for support services (e.g., special educators, aides).
5. **Inadequate Teacher Training Barrier**- Many schools lack teachers trained to handle persons with disabilities.

Strategies for Implementing Education Component in CBID

- o **Involvement and Awareness:** Engaging the entire community in understanding disability and the importance of education.
- o **Collaboration with Educational Institutions:** Building partnerships with the Ministry of Education, early childhood department, learning institutions, CSO, DPO and other stakeholders in the education sector to support inclusive educational practices.
- o **Individualized Education Plans (IEPs):** Developing tailored educational plans for students with disabilities to accommodate their unique learning needs.

- o **Teacher Training and Support:** Providing training programs for educators to enhance their skills in supporting students with diverse needs.

TOPIC 3.2 EARLY CHILDHOOD CARE AND EDUCATION

Module outcome

By the end of the secession the learner should be able to: -

- o Define early childhood care education
- o Identify the benefits of early childhood education
- o Outline the barriers to early childhood care and education in Kenya
- o To provide other avenues of inclusive early childhood education both at the family and community levels

Notes for Facilitator

Step 1: The facilitator defines what the term early childhood care and education

Step 2: Learners brainstorm on the benefits of early childhood care and education

Step 3: In groups, learners identify barriers to early childhood education and other avenues of promoting inclusive early childhood education

Step 4: Learners present and the facilitator provides professional input on the barriers to early childhood care education

Definition

Early childhood care and education

This term refers to education from birth until the start of formal primary education. It takes place in formal, non-formal and informal settings, and focuses on child survival, development and learning – including health, nutrition and hygiene. Early childhood education sows the seeds for an inclusive society, because it is where children with and without disabilities can learn, play and grow together.

3.2.1 Benefits of Early childhood education

- o It gives children the best start in life by providing the best opportunities to learn and develop.
- o Helps children to make friends and develop independence and learn new routines
- o It supports their transition to school from one level to the other
- o Early childhood education offers screening for health and behavioral issues which assist in early identification of health conditions

3.2.2 Barriers to early childhood education in Kenya

- o **Inadequate teaching and learning resources :** many ECDE centers lack adequate teaching and learning resources and facilities suitable for ECDE in the learning environment. These include properly ventilated classrooms, furniture, kitchen, clean safe water, playground, toilets and play materials suitable for children.
- o **Financial constraint :** which leads to ineffective implementation of early childhood education programmes and activities.
- o **High Teacher – Child ratio with poor remuneration:** Teacher- child ratio has been a subject of attention among researchers in relation to factors facing teaching the learning processes.

3.2.3 Support early childhood education at home and in the community

At family level

Early childhood care and learning begin at home; therefore, family involvement is essential. Parents play a crucial role in the early years, they influence their child's development through their attitudes and behavior, e.g. breastfeeding and spending time stimulating and playing with their child.

At community level

Different types of early childhood education opportunities are available in the community, e.g. playgroups, day centers, mother and child groups and women's self-help groups with crèches (childcare). The inclusion of children with disabilities in their communities can be promoted through simple activities, such as:

- o Encouraging families to take their children on outings into the community
- o Encouraging families to allow children to play outside the house, with support from special seating or assistive devices if needed, which the community can help to develop
- o Encouraging children with and without disabilities to play together
- o Involving the local community in making the environment more accessible and welcoming, including the preschool and local playground.

TOPIC 3.3 PRIMARY EDUCATION

Module outcome

By the end of the session the learner should be able to: -

- Define primary education and key concepts
- Identify ways of promoting inclusive primary education

Notes for facilitator

Step 1: Learners defines key concepts in primary education

Step 2: Learners brainstorm on ways of promoting inclusive primary Education

Step 3: Learners present on plenary as the facilitator provides input on ways of promoting inclusive primary Education

3.3.1 Definition of key concepts in Primary education

Primary education usually begins around the age of six or seven years and continues into the early teens. Primary education is the gateway to higher levels of education and is a high priority in the context of development.

Whole community approach

It is the responsibility of the whole community to promote and support inclusion, and local primary schools provide a key opportunity and environment in which to demonstrate this. CBID personnel have many responsibilities and are not specialists in education, therefore they need to work in collaboration with the community (schools, families, persons with disabilities, community leaders), focusing on raising awareness about rights, inclusion and the social model, and mobilizing and supporting all those involved.

Whole-school approach

The whole-school approach ensures that managers, head teachers, class teachers, administrators, matrons, caretakers, parents, children with and without disabilities, and any others who have a connection with the school will work together to raise awareness about disability and identify and remove barriers to facilitate the inclusion of disabled children in the local school.

Social inclusion

Learning to live together with others, to relate to those people who are different, and to be open, helpful and respectful of others is just as important as learning academic skills. Children who experience difficulty in learning academic skills may benefit from being included socially in the local school environment. This also prepares all children to become members of an inclusive society.

Responding to diversity

Children are all different and learn in different ways. Schools need to respond to this diversity by ensuring that curricula, teaching methods and environments are flexible and accommodating to all. A flexible system requires adjustments to be made for everyone, not just one particular group.

Welcoming and accessible environment

The school environment needs to be physically accessible for all children, with specific attention given to ensuring toilets are accessible for children with disabilities. Attention to the appearance and atmosphere of the school ensures that the environment is welcoming for all.

Learner-centered approach

A learner-centered or child-centered approach means that all processes and structures in a school are centered on supporting each child to learn and participate. Too often, the teacher, rigid curriculum or fixed timetable becomes the center of attention, regardless of whether the child is actually learning.

Flexible primary education

Primary education usually takes place within a designated building, but this is not essential. It is important not to have a fixed idea about the role of schools. Some children, e.g. those with severe or multiple impairments, can still be included in primary education, even when they are receiving their education at home (see non-formal education). Inclusive education does not literally mean putting every single child with a disability into a school building.

Suggested Activities that promote Inclusive Primary Education

- o **Mobilize the community** - Situations regarding primary education differ from community to another. Some local primary schools may be supportive of inclusive education, while others may not be. Sometimes there may be no primary school at all. Some communities may have a tradition of sending children with disabilities away to special schools or special units. Whatever the situation, an important starting point is to raise awareness about inclusive education in the communities and gain their support. Awareness creation can be done Radio, public meetings, posters, involving organization of persons with disabilities and child to child activities in school.
- o **Support and involve families** - CBID personnel can have a major role in supporting and involving families in the process of enabling the local school to be inclusive. Teachers often need to learn that parents are partners in education.
- o **Support the child with disability** - Many children with disabilities are prevented from accessing primary education because of poor health status and environmental barriers, e.g. large distances between home and school and inaccessible public transport facilities. Improvements in health and physical access can be achieved with appropriate medical care, rehabilitation and assistive devices CBID personnel need to ensure that children can access these services.

- o **Help make the school welcoming and accessible** - CBID personnel can involve teachers, students and families in identifying and addressing accessibility issues within school environments. It is important to recognize that accessibility includes more than just ramps for children using wheelchairs.
- o **Help create a learning environment** - This refers to all the activities and approaches that need to be in place so that all children have the opportunity and support to achieve their highest potential.

TOPIC 3.4 SECONDARY AND HIGHER EDUCATION

Module outcome

By the end of the session the participants will be able to: -

- o Define Secondary education, Higher education and key concepts
- o Identify ways of promoting inclusive Secondary and higher education

Facilitators Notes:

Step 1: The facilitator defines key concepts in Secondary and Higher education

Step 2: The participants brainstorm on ways of promoting inclusive secondary and higher education

Definition of Key Concepts

Secondary education refers to the stage of formal education that follows primary education and precedes higher education whereas **Higher education** is education, training, and research guidance that takes place after the post-secondary level. In poor communities, only a few children have access to secondary and higher education, i.e. education beyond the compulsory level, and students with disabilities are either totally excluded or face constant battles to prove their abilities.

Changing the system: As in the other education elements, the concept of working to change the system to fit the student is very important. However, secondary and higher education systems can be very rigid and controlled at the national government level, making it challenging for communities and CBID Programmes to have any influence.

The barrier of low expectations: A major barrier in further education for students with disabilities is the low and limited expectations of others in relation to their interests, abilities and ambitions. So, perceptions and expectations about persons with disabilities need to change. Around the world, persons with disabilities are today reaching the top of a very wide range of professions and gaining the highest qualifications.

Achievement and assessment: Achievement will have different meanings for each student, depending on his/her capabilities and potential. For example, for a student with an intellectual impairment, achievement may mean successfully developing skills for independent living, social skills and practical numeracy and literacy skills, while for others achievement may be related to academic tasks. Technical, vocational, artistic and creative skills should be valued as much as academic ability.

Suggested Activities that promote Inclusive Primary Education

- o **Involve the community-** This can be challenging because often secondary and higher education facilities are a long distance from the community. CBID Programmes can contribute by; encouraging the community and school authorities to organize transport facilities, mobilizing the community to raise finances for fees, uniforms, assistive devices and additional support, and encouraging them to recognize students with disabilities as a resource for the community; gaining access to grants and government loans, and funding from donor agencies, facilitating lobbying and formation of advocacy groups together with the local organizations of and for persons with disability to promote the rights of students with disabilities in secondary and higher education.
- o **Support the family-** Some families may be quite resistant to the idea of students with disabilities having access to secondary or higher education, owing to overprotection or lack of belief in their abilities, resentful siblings, and for females, lack of value placed on their education. There can be many doubts, difficulties and adjustments that families need to address. CBID personnel can educate and support families so that they offer support to students with disabilities, both during their education and in transitional periods.
- o **Help to create an inclusive learning environment-** Many of the activities listed in the Primary education element are also common to secondary and higher education.
- o **Encourage best use of specialist resources and support -** CBID Programmes can encourage schools and colleges to use specialist resources correctly, so increasing the inclusion of students with disabilities rather than increasing their stigmatization or segregation.
- o **Help to ease transitions -** Transitional periods during education include: primary to secondary, secondary to tertiary, and tertiary to a sustainable livelihood. Often, students with disabilities need to move away from their communities to complete higher education. This can make transitional periods more difficult, and many find it very challenging without the support of their families and communities

TOPIC 3.5 NON- FORMAL EDUCATION

Module outcome

By the end of the session the learner should be able to: -

- o Define non-formal education
- o Identify core principles of non-formal education
- o Identify activities that can make non-formal education inclusive

Notes for Facilitator

Step 1: The facilitator defines non-formal education

Step 2: In groups participants identify the core principles of non-formal education

Step 3: The participants brainstorm on ways of promoting inclusive secondary and higher education

What is non-formal education

Non-formal education refers to education that occurs outside the formal school system. Non-formal education is often used interchangeably with terms such as community education, adult education, lifelong education and second-chance education. It refers to a wide range of educational initiatives in the community, ranging from home-based learning to government schemes and community initiatives.

3.5.1 Core principles of non-formal education

- o **Relevant** to the learner's life and the needs of society, and will be so in the future. Mechanisms for involving children, parents and local communities as well as educators in deciding the content of what is taught will ensure that non-formal education is relevant to the needs of communities and draws on local resources and personnel.
- o **Appropriate** to the level of the learner's development, with new content and experiences being introduced when the learner is ready. Teaching is learner centered and student-directed.
- o **Flexible** in what is taught and how it is taught, and to the needs of the different learners, e.g. adults and children who work, who live on the street, who are sick, who are in prison, who have a disability or who are victims of conflict or emergency, and flexible to traditional/indigenous learning styles.
- o **Participatory** in that learners are active participants in their learning, and that they and their families and communities are involved in running the non-formal education programme.
- o **Protective** of children from harm, and protective of their rights to survival and development. Places of non-formal education should be healthy and safe, and provide proper nutrition, sanitation and protection from harm.
- o **Inclusive** of all children regardless of background or ability, respecting and utilizing the differences between them as a resource for teaching and learning. Non-formal education often targets marginalized groups, e.g. nomadic communities, girls, persons with disabilities, school dropouts and working children. For students with disabilities and other marginalized groups, non-formal education is very helpful, responding to and fitting their needs.
- o **Quality:** non-formal education Programmes have the potential to be of exceptionally high quality, because they can respond more easily to the needs of individuals and specific groups in the community.

3.5.2 Suggested Activities that promote Inclusive Non-formal education

- **Help make existing non-formal education Programmes inclusive** - A wide range of non-formal education Programmes may already be operating in the community. These Programmes may be oriented towards literacy and basic education, health promotion (reproductive health issues, sexually transmitted diseases, HIV/AIDS), environmental issues, agriculture, fishing, rural development and/or community development. Non-formal education programs offer excellent opportunities for persons with disabilities to be educated alongside their non-disabled peers. CBID Programmes can identify existing non-formal education Programmes and encourage them to become inclusive rather than establish parallel Programmes for people with disabilities. Making non-formal education Programmes inclusive will involve encouraging the enrolment of

people with disabilities in all types of Programmes, and ensuring that teaching is conducted in accessible places and that teaching formats are accessible

- **Support home-based learning** - Home-based learning can be supplementary, preparatory or an alternative to formal education. It can be relevant for students with extensive needs, for whom the priority is to learn basic daily living skills, basic communication and basic functioning. These skills are best learned in environments where they will be used rather than in artificial environments. CBID personnel liaise closely with family members, teachers and students, carry out home visits on a regular basis and work with the entire family to ensure successful home-based learning.
- **Sustain specific learning group** - Sometimes there is a specific learning need (such as learning sign language or Braille) that requires students to come together in their own groups to study. CBID personnel can provide assistance in developing and sustaining these groups, and can link students with disabilities with disabled people's organizations, which can be a useful resource to facilitate their learning.
- **Ensure community-based day centers are appropriate** - Community-based day centers are often established to provide respite for parents who have children with disabilities needing 24-hour care. While these centers may be a great support for parents, often the quality of activities and learning for children is very poor. CBID Programmes can help ensure that the centers provide age-appropriate play and activity-based learning, and that the "best interests of the child" are the priority. The centers should also be as inclusive as possible, and involve local children without disabilities and their parents.
- **Facilitate links with formal schooling** - non-formal education Programmes tend to use different and more flexible curricula and teaching methods, which may enable learners to benefit, but can also jeopardize a student's transition into the formal system. Without systematic links between the formal and non-formal education systems, non-formal education may contribute to the segregation of persons with disabilities.
- **Ensure the curriculum is practical and relevant** - non-formal education curricula often have greater flexibility and can be easily adapted to suit the needs of individuals.

TOPIC 3.6 LIFELONG LEARNING

Module outcome

By the end of the session the learner should be able to: -

- Define key concepts in lifelong learning
- Identify activities that promote inclusive lifelong learning

Notes for facilitator

Step 1: The facilitator defines key concepts in lifelong learning

Step 2: In groups participants identify activities that promote inclusive lifelong learning and make presentation

3.6.1 Definition of key concepts

Lifelong learning

Lifelong learning refers to all purposeful learning activities undertaken on an ongoing basis throughout

a person's life, with the aim of improving knowledge, skills and competencies. Learning is no longer understood to be divided into a time and place for gathering knowledge (i.e. school) and a time and place to apply the knowledge (i.e. work). Without lifelong learning, the capacity of individuals and communities to renew their knowledge and skills and adapt to political, economic, environmental, technological or social changes is considerably reduced. Lifelong learning can take place in a variety of settings, both formal and non-formal. It is complementary to other forms of education, and continues even when formal education or training opportunities are available.

Adult learning

Adult learning is a vital component of lifelong learning and is essential for employability, social inclusion, active citizenship and personal development. Adults learn differently from children, and their specific needs and ways of learning need to be taken into consideration in the design of teaching methods and materials. Lifelong learning opportunities need to be learner-centered, with a focus on personal goals, past life experiences and promotion of positive self-esteem.

Open learning

Open learning often occurs in “open schools” and delivers learning opportunities where, when and how the learner needs them, and provides a more relevant curriculum for older learners who have never had the chance to attend or complete formal education at school. Teaching methods are usually unconventional and creative, and include the use of information and communication technology. Students will often study specially designed teaching materials in their own homes or workplaces, or wherever is convenient for them at a pace that suits them.

Life and survival skills

Life and survival skills refer to the knowledge and skills that everyone needs in order to function and participate effectively at home, in the community and in wider society. Because of exclusion and discrimination, persons with disabilities especially need these skills, and support for developing them is an essential part of lifelong learning at any stage of life.

3.6.2 Suggested activities to ensure inclusive Lifelong Learning

- o **Facilitate support for transitions-** CBID personnel are ideally placed to liaise between students with disabilities and families, employers, facilitator and self-advocacy groups. The transition from school to work or to further skills training and increased independence can be very challenging if unsupported. CBID personnel can help identify and advise on available careers and peer counselling opportunities and ensure that these are accessible, relevant and effective.
- o **Identify opportunities for continuing education -** Continuing education opportunities are increasingly becoming available through local educational institutions, as well as through distance education and Web-based educational Programmes. Distance education can often eliminate barriers such as distance, lack of accessible transport and a hostile environment, as well as enabling people with disabilities to benefit from higher education. CBID personnel need to be aware of these developments and be equipped to help persons with disabilities to take advantage of the benefits they offer. CBID Programmes can identify opportunities for continuing education, especially through distance learning, and can work with institutions to help them enroll students with disabilities.
- o **Ensure opportunities for learning life and survival skills –** CBID Programmes can liaise with organizations and community groups to provide opportunities for youth and adults with disabilities to develop the skills required for daily life and participation in society, such as those related to self-care, transport, shopping, appropriate social behavior, assertiveness and self-esteem, sex-

uality, marriage and parenting, HIV/AIDS awareness, maintaining health and well-being, financial management, citizenship and political participation. Again, the key principle is to identify existing education Programmes and work to make them inclusive rather than set up parallel Programmes.

- o **Work with educators in the community to promote social inclusion-** Exclusion is more often the result of external factors than personal factors. Community educators – in government, non-governmental organization and other community schools, and educational institutions including private and commercial schools – can bring about the inclusion and active participation of people with disabilities in lifelong learning activities once they become aware of the need. CBID Programmes can work with community organizations, especially organizations of and for persons with disability and parents' associations, to share their experiences and knowledge to create disability awareness.

MODULE 4 : LIVELIHOOD COMPONENT

Module description

This module will cover the key concepts of livelihood and cites the importance of persons with disabilities gaining access to employment opportunities, financial resources and social protection.

Module Outcomes:

By the end of the session the learners will be able to: -

- o Describe key concepts in livelihood in relation to CBID
- o State key elements under livelihood component
- o State the goal of livelihood in relation to persons with disabilities.
- o Describe the role of CBID in relation to livelihood and its desirable outcomes
- o State the importance of livelihood.

Module Topics

SN	Topic Names	Duration (Minutes)
4.1	Introduction to Livelihood Component	35
4.2	Skills Development	30
4.3	Wage employment	60
4.4	Self- employment	60
4.5	Financial services/literacy	60
4.6	Social Protection	30
	Total	275

TOPIC 4.1: INTRODUCTION TO LIVELIHOOD

Methodology:	Flip Charts, Marker pen, Masking tape, Group discussions
Time:	35 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: What does livelihood mean to you? (5 min) 2. Present the livelihood definition and briefly explain the key concepts (10 min) 3. Divide participants into 4 groups to discuss the key elements in livelihood component and make presentation (10 min) 4. Summarize on the key elements in livelihood component (5 minutes) 5. Present the importance of livelihood (5 min)	

4.1.1 Definition

Livelihood is a means by which a person or a household secures their basic necessities of life and sustains themselves. It encompasses peoples' capabilities, assets, income and activities required to secure necessities of life.

Livelihood is part of CBID because "It is essential to ensure that both youth and adults with disabilities have access to training and work opportunities at community level". A CBID programme that does not address the skills development and livelihood needs of youth and adults with disabilities in a community is incomplete and limits the sustainability of other efforts. The learning of knowledge and skills begins in the family at an early age as children watch and learn how to do things from parents and other family members. Children with disabilities should also be encouraged to learn, participate and make a contribution in the family.

4.1.2 Legal Framework

Access to decent work and livelihood opportunities is a core right under Article 27 of the CRPD, which mandates equal employment rights, access to vocational training, and entrepreneurship support. In Kenya, relevant frameworks include:

- o Persons with Disabilities Act (2025) – provides for a 5% employment reservation for persons with disabilities in public institutions/ provision of reasonable accommodation in workplace.
- o Employment Act 2007 – nondiscrimination in employment in account of disability.
- o Micro and Small Enterprises (MSE) Policy (2020) – encourages inclusive entrepreneurship and access to finance for persons with disabilities.
- o National Employment Authority Act (2016) – supports fair recruitment and training access.
- o SDG 8 – promotes decent work and economic growth for all.

CBID livelihood initiatives should advocate for disability inclusion in local employment schemes, vocational training, access to credit, and self-employment programs, ensuring persons with disabilities are not excluded from economic empowerment efforts.

4.1.3 Key Concepts

Work

Work is an important life activity that contributes in maintaining the individual, the family and the household by providing services and/or goods for the family, the community and society at large. Most importantly, work provides opportunities for social and economic participation, which enhances personal fulfilment and a sense of self-worth.

Decent work

Not all types of work are desirable; it is important to distinguish between decent work and work which exploits and perpetuates poverty and lack of dignity. Decent work is work which dignifies and does not demean. Scarcity of work may mean that many poor people are not in a position to choose how they earn a living and accept working conditions that are far from decent.

Environmental accessibility

Lack of environmental accessibility is a major barrier for persons with disabilities in all low-income countries: inaccessible public transport, workplaces and communications mean that it may be impossible for persons with disabilities to get to work and do their job.

The formal and the informal economy

The formal economy is regulated by the Government and includes employment in the public and private sectors, where workers are hired on contracts with a salary and receive benefits such as pension schemes and health insurance. The informal economy is the unregulated sector of a country's economy. It includes small-scale agriculture, petty traders, home-based enterprises, small businesses employing a few workers and a multitude of similar activities.

Lifelong learning

Education and the acquisition of skills should not be something that happens only in childhood. For both children, youth and adults with disabilities, continuing education and upgrading of skills is important in order to create, sustain and develop livelihood opportunities. The need is for education for life and education throughout life, a concept that is now referred to as lifelong learning.

Rural and urban

The continuum of human settlements ranges from large cities at one end to small, remote villages at the other. In between are large villages, villages close to towns, small towns, shanty towns which are part of a city complex and "peri-urban areas" (populated areas close to, but not necessarily part of, major towns or cities). In this component, "urban" means relating to towns or cities and their peri urban areas and "rural" means relating to villages, usually small and mainly dependent on agricultural activities.

Opportunities for earning an income are very different in rural and urban areas. In urban areas, there is a vast range of types of employment, both in the formal and informal economies. In rural areas where the economy is based on small-scale agriculture, there are less employment options.

Focus on the whole family and community

Disability is not just an individual issue; it affects the entire family and community as well. Families in poor communities usually survive through several sources of income. A family member with a disability can make a contribution to family life and livelihood, and therefore activities to support livelihoods must take the whole family into account.

Aspirations and role-models

Some persons with disabilities are caught in a damaging circle of low expectations and low achievement. They often aim at, or are channeled into, work and occupations below their potential, for the simple reason that there are limited expectations of what they can do. However, people with disabilities who have broken free of these limited expectations and have succeeded in fulfilling their potential exist in many communities; they can be used as role-models to enable other people with disabilities to raise their own aspirations.

4.1.4 Key elements in the livelihood component.

Skills development

Skills are essential for work. There are four main types of skills: foundation skills, technical and professional skills, business management skills and core life skills. These skills can be acquired through traditional home-based activities and education, in mainstream vocational training centers and as an apprentice with members of the community.

Self-employment

Self-employment provides the opportunity for persons with disabilities to earn a livelihood. Self-employment activities involve production, providing a service or trading; they may be individual or group, part-time or full-time; they are equally applicable in both rural and urban areas, in both the formal and the informal economy. Self-employment provides income for large numbers of women and men with disabilities and a chance to contribute economically to their families and communities.

Wage employment

Wage employment means any salaried or paid job under contract (written or not) to another person, organization or enterprise. It is more likely to be in the formal economy, but may also be in the informal economy. Persons with disabilities face many barriers to finding decent wage employment. However, there has been encouraging developments in many businesses, which have proactively sought to employ persons with disabilities. CBID Programmes can help overcome or lessen barriers to wage employment.

In Kenya, persons with disabilities face significant barriers in accessing decent wage employment, including discrimination, inaccessible workplaces and transportation, and a lack of education and training opportunities. These challenges are compounded by negative employer attitudes, a lack of awareness about disability, and inadequate support systems.

Financial Services and financial literacy

Persons with disabilities have the same needs for financial services as persons without disabilities, both to start and to develop businesses and to manage their lives generally. Microcredit refers specifically to loans and the credit needs of clients, while microfinance covers a broader range of financial services, e.g. savings, insurance, housing loans and remittance transfers. Microfinance schemes are available from cooperatives, village banks, savings and credit associations, self-help groups, commercial banks and microfinance institutions. Important at the community level is the self-help groups.

Social Protection

Social protection measures are intended to provide a safety net to protect people against extreme poverty \ lack of income through illness, disability or old age. Persons with disabilities have an equal right to the social protection measures available to citizens generally. Because they are amongst the poor and most marginalized, persons with disabilities must be effectively included in all social assistance Programmes. In Kenya these include the consolidated cash transfer (Inua Jamii Program) which targets persons with severe disabilities, older persons, orphans and vulnerable children. The hunger safety net program and social economic inclusion programs.

4.1.5 Importance of livelihood

- o By encouraging and facilitating work by women and men with disabilities, Community Based Inclusive Development (CBID) Programmes can help individuals and their families to secure the necessities of life and improve their economic and social situations.
- o By taking into consideration the needs and views of people with disabilities and making provision for their inclusion in national poverty reduction and other development Programmes, opportunities for education, skills acquisition and work can be provided for people with disabilities and their families, enabling them to emerge from poverty. Accessing livelihood opportunities is one of the key factors in eliminating poverty.
- o Livelihood is part of CBID because “It is essential to ensure that both youth and adults with disabilities have access to training and work opportunities at community level” A CBID programme that does not address the skills development and livelihood needs of youth and adults with disabilities in a community is incomplete and limits the sustainability of other efforts
- o The Livelihood component, like every other component of the CBID matrix, has very strong linkages with the other components. There are necessary linkages between efforts to promote and facilitate livelihood in CBR and efforts to enhance access to health care, education services and social opportunities. An individual with a disability needs to be healthy and may need an assistive device in order to work.

- o Future work opportunities are greatly enhanced for children, youth and adults who have access to primary and secondary education, as well as opportunities for skills training. Likewise, a person with a disability who is working is empowered and better able to obtain the necessities of life, maintain a family and participate actively in the social, cultural and political life of his/her community.

TOPIC 4.2: SKILLS DEVELOPMENT

Methodology:	Flip Charts, Marker pens, ICT output devices, masking tape, group discussions
Time:	55 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: What does skills development mean to you? (5 min) 2. Present the skills development definition and briefly explain the key concepts (10 min) 3. Divide participants into groups of 5 to discuss ways by which persons with disabilities can learn and develop the knowledge, skills and attitudes necessary to earn their livelihood in the community and make presentations. (15 min) 4. Summarize on the key points on ways to acquire skills (5 min) 5. In plenary, identify the learning institutions or places for on -the- job training in the community (5 min) 6. In pairs, describe the activities to undertake to improve skills development (10 min) 7. Facilitator wraps up by providing input (5 min)	

4.2.1 Introduction

Persons with disabilities need skills to engage in livelihood activities. However, they start with a number of disadvantages. They often lack access to basic education, making them unqualified to join skills training courses. These disadvantages frequently result in a lack of skills, as well as low confidence, expectations and achievement. Different types of skills are required for successful work. They include foundation skills acquired through education and family life, technical and professional skills which enable a person to undertake a particular activity or task, business skills required to succeed in self-employment and core life skills, including attitudes, knowledge and personal attributes.

4.2.2 Key Concepts

Foundation skills are those acquired through basic education and family life. They include, for example, literacy, numeracy, ability to learn, reasoning and problem-solving. These types of skills are needed for work everywhere, in all contexts and cultures, in both formal and informal economies.

Technical, vocational and professional skills are those which equip someone to undertake a particular task – how to produce or repair something, or provide some kind of service. Examples are carpentry, tailoring, weaving, metalwork, lathe operation, basket making, tin smithing, shoemaking. More advanced technical skills, such as engineering, medicine, physiotherapy and computer technology, are normally referred to as professional skills. Generally, the more advanced the techniques, the higher the educational level required and the more formal the training, often taking place in technical institutions and resulting in formal certification of competence.

Business skills (also called entrepreneurial skills) are those required to succeed in running a business activity. They include money and people management, as well as planning and organizational skills. They also include risk assessment, market analysis and information-gathering, business plan preparation, goal-setting and problem-solving. These skills usually require a basis of numeracy and literacy. Training courses which teach trades likely to lead to self-employment, such as carpentry, radio repairs, two-wheel motor mechanics and weaving, have an obligation to teach business skills alongside the technical skills.

Core life skills consist of the attitudes, knowledge and personal attributes necessary to function in the world. They include how to relate to customers, how to present oneself, effective listening and communication, creative thinking and problem-solving, personal management and discipline, interpersonal and social skills, the ability to network and work in a team, and work ethics.

4.2.3 Ways to acquire skills

There are a number of ways by which persons with disabilities can learn and develop the knowledge, skills and attitudes necessary to earn their livelihood.

They include:

- o Self-effort;
- o Home-based skills acquisition within the family;
- o Basic education;
- o Vocational training in schools;
- o Community-based training, including formal or informal apprenticeships with individuals in the community;
- o Training in mainstream vocational training centers or vocational rehabilitation centers
- o On-the-job training or apprenticeship;
- o Training courses at college and university;
- o Participation in small-enterprise development programmes that include basic business skills training, business development services and mentoring;
- o Training by employers.

The choice of the most appropriate method of skills development depends upon the interests, capabilities and resources of the individual, as well as the opportunities and support available in his/her community.

TOPIC 4.3: WAGE EMPLOYMENT

Methodology:	Flip Charts, Marker pen, Masking tape, Group discussions, assorted stationery
Time:	40 minutes
Facilitator Instructions:	
1. Give a brief introduction on wage employment and explain the key concepts (10 min) 2. Divide participants into 4 groups to discuss the barriers in wage employment and possible ways of addressing the barriers (15 min) 3. Ask participants to present what they have discussed (10 min) 4. Facilitator wraps up the session by providing input (5 min) 5. Facilitate a discussion with the following question: Describe the activities to undertake to improve wage employment amongst persons with disabilities. (10 min)	

4.3.1 Introduction

Everyone has a right to decent work, and persons with disabilities are employed across all sectors and types of organizations. There are no jobs exclusively “best suited” for them; rather, like all job seekers, their suitability for employment depends on their unique interests, goals, skills, abilities, and education. When assisting people with disabilities in finding appropriate work, it’s crucial to consider these individual factors alongside labor market demands and available support. Wage employment should always be a viable option for individuals with disabilities seeking work, and they should be considered for employment opportunities. Access to wage employment should always be considered an option for persons with disabilities looking for work because of their skills and their contributions to the workplace, and not out of charitable concern. For this reason, education and comprehensive skills training is of vital importance.

Role of CBID

The role of CBID is to enable persons with disabilities to access and retain wage employment, by working to increase equal access and treatment in the workplace, as well as access to services that lead to wage employment.

4.3.2 Key concepts

Many countries, including Kenya have adopted laws and policies that promote the employment of persons with disabilities in regular jobs, including quotas, anti-discrimination measures, positive employment measures, job retention or return-to-work measures and alternative employment policies.

Wage employment options- The two main forms of wage employment available to persons with disabilities are as follows: -

Mainstream employment

Persons with disabilities have the right to participate in the mainstream, competitive labor market, with the same benefits and wages as other workers. Employment opportunities exist within the government sector, non-governmental sector and private sector, including multinational companies and businesses in both the formal and informal economy.

Supported employment

Some persons with disabilities may require ongoing support to be able to participate in a mainstream employment workplace. There are two main types of supported employment:

- o Individual placement with support, where a support worker provides on-the-job training as well as the support the worker with disabilities needs to do his/her job effectively;
- o Work crews, where a group of persons with disabilities is supported to work together either in a mainstream setting or as a mobile work crew that provides contracted services in the community, such as office cleaning, gardening, outdoor maintenance and car cleaning.

4.3.3: Barriers in wage employment

The Specific Barriers are:

1. Discrimination and Stigma:

Negative attitudes from employers, colleagues, and the public create a hostile environment, with some viewing persons with disabilities as less competent or a burden.

2. Inaccessible Workplaces:

Physical barriers, lack of reasonable accommodations, and inaccessible transportation systems hinder access to and from workplaces.

3. Lack of Education and Training:

Limited access to quality education and vocational training restricts the pool of qualified candidates with disabilities.

4. Negative Employer Attitudes:

Employers may harbor misconceptions about the capabilities of persons with disabilities, leading to reluctance to hire them or provide equal opportunities.

5. Limited Awareness:

A lack of awareness about the potential of persons with disabilities and their rights creates a barrier to inclusion in the workplace.

6. Inadequate Support Systems:

Lack of access to healthcare, social support, and financial resources further exacerbates the challenges faced by persons with disabilities in finding and maintaining employment.

7. Communication and Information Barriers:

Difficulty accessing information about job opportunities and navigating the application process can also be a significant hurdle.

8. Economic Exclusion:

Persons with disabilities are often relegated to lower-wage or informal sectors, lacking job security and opportunities for advancement.

Addressing the Barriers in wage employment:

To improve this situation, a multi-faceted approach is needed, including:

1. Promoting Awareness:

Raising public awareness about the capabilities of persons with disabilities and challenging negative stereotypes.

2. Improving Education and Training:

Expanding access to quality education and vocational training programs tailored to the needs of persons with disabilities.

3. Enacting and Enforcing Anti-Discrimination Laws:

Strengthening and enforcing laws that prohibit discrimination based on disability.

4. Creating Accessible Workplaces:

Ensuring that workplaces are physically accessible and providing reasonable accommodations to enable people with disabilities to perform their jobs effectively.

5. Promoting Inclusive Hiring Practices:

Encouraging employers to adopt inclusive hiring practices and providing support for employers who hire persons with disabilities.

6. Developing Support Systems:

Strengthening healthcare, social support, and financial assistance programs to address the needs of persons with disabilities.

7. Encouraging Self-Advocacy:

Supporting organizations of persons with disabilities to advocate for their rights and promote their inclusion in the workforce.

TOPIC 4.4: SELF-EMPLOYMENT

Methodology:	Flip Charts, Marker pen, ICT output devices, Masking tape, Group discussions.
Time:	25 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: What is self-employment? (5 min) 2. Present the self-employment definition and briefly explain the key concepts (10 min) 3. Discuss in plenary activities that can enhance self-employment amongst persons with disabilities (10 min)	

4.4.1 Definition

In most low-income countries, the informal economy offers more opportunities for livelihood than the formal economy. This is especially so for persons with disabilities, who may be blocked from wage employment in the formal economy either by lack of education and other qualifications or by negative attitudes among employers.

In the informal economy, self-employment either alone or in a group is the most likely way to earn an income. Self-employment activities include making a product (e.g. potato crisps, clothing, furniture), providing a service (e.g. hairdressing, massage, repairing two-wheel vehicles, running a cyber-cafe) and selling goods (e.g. running a shop, restaurant or stall).

Self-employment is a term applied to economic activities in both the formal and informal economies which are owned, operated and managed by an individual or a group.

Role of CBID

The role of CBID is to encourage and support self-employment by assisting persons with disabilities and their families, either individually or in groups, to access skills development as well as financial and material resources.

4.4.2 Key Concepts

There are three broad categories:

1. Income-generating activities;
2. Small and medium enterprises;
3. Self-help groups and group enterprises.

Whatever their size, these businesses are involved in one or more of three types of activity: production, providing a service, or trading.

Income-generating activities

Income-generating activities or programmes are small-scale activities that may be the sole source of income for an individual or group or supplement another source of income, such as farming. The activities may be full-time, part-time and/or seasonal and are usually based on traditional technologies, local materials and local markets. They often take place in rural areas and are normally part of the informal economy.

Small and medium enterprises

Small and medium enterprises operate on a larger scale than income-generating activities. They employ more than one person and constitute the principal source of income for those engaged in them.

Self-help groups and group enterprises

The formation of self-help groups has become in some countries the dominant tool for community development and poverty reduction. “Self-help group” is the generic term for a group of people who come together for a common purpose. In many cases, this is to save money through a group savings scheme. Savings may be used for helping individual group members to start or expand a business, or the group may run a business as a joint enterprise to share the risk and undertake an activity on a scale impossible for an individual. But the most important function of such a group is that it brings people together for a common purpose, strengthens social bonds and makes community development a reality. Persons with disabilities may join nondisabled groups, or form groups of their own.

4.4.3 Barriers to self-employment

- o Limited access to financial services: Many persons with disabilities struggle to secure loans or credit due to lack of collateral, discriminatory lending practices, or absence of tailored financial products.
- o Inaccessible infrastructure: Physical environments such as marketplaces, transportation, and business premises often lack accommodations like ramps or accessible restrooms.
- o Stigma and discrimination: Negative societal attitudes often lead to exclusion from business networks, mentorship opportunities, and customer bases.
- o Limited access to training and education: Vocational and entrepreneurial training programs are often not inclusive or adapted to diverse needs.
- o Lack of awareness of opportunities: Persons with disabilities may not be informed about available resources, grants, or support systems for entrepreneurs.
- o Digital divide: Limited access to technology and internet connectivity restricts participation in online business and e-commerce.

Addressing the barriers

- o Access to microfinance, youth enterprises funds and savings groups will assist in raising capital
- o Promote the use of universal design principle to ensure environments are accessible to all
- o Awareness campaigns-conduct targeted campaigns to educate the public
- o Training and mentorship and business development services has a key role for effective operations
- o Information and navigation create accessible step by step guides specific for persons with disabilities
- o Establish mentorship and support networks and link persons with disabilities

TOPIC 4.5: FINANCIAL SERVICES (LITERACY)

Methodology:	Flip Charts, Marker pen, Masking tape, Group discussions
Time:	55 minutes
Facilitator Instructions:	
1. Introduce the topic on financial services and briefly explain the types of financial services (10 min) 2. Divide participants into 4 groups to discuss the types of financial services and make presentations (10 min) 3. In the same groups, participants brainstorm on activities to undertake to improve access to financial services amongst persons with disabilities. (10 min) 4. Introduce a roleplay into the four groups and make presentations (15 min) 5. In plenary, discuss questions on the role play (5 min) 6. Facilitator to wrap up the session (5 min)	

4.5.1 Definition

Financial services include savings, credit, grants, insurance and money transfer services. Microcredit refers specifically to small-scale loans and the credit needs of clients, while microfinance covers a broader range of financial services, for example savings, insurance, housing loans and remittance transfers, available to individuals and small-scale businesses. A local microfinance institution may offer microfinance plus development activities, such as entrepreneurial and life skills training and advice on topics, e.g. health and nutrition, sanitation, improving living conditions and the importance of educating children.

Role of CBID

The role of CBID is to identify, facilitate and promote access of persons with disabilities to financial services.

4.5.2 Types of Financial Services

There are five main types of financial services:

Savings

Saving is a habit which is based on a value system of self-reliance. Regular saving of even very small amounts gives a person struggling to survive the practical opportunity to learn to manage money. It creates financial capital, increases feelings of individual self-worth and group solidarity and establishes credit worthiness for access to financial services. Nobody should be considered too poor to save, and having a disability should also not be used as a reason for not saving.

Savings can be used to invest in education, training or business activities. Most new business activities require some kind of personal savings. Lending institutions will normally not look favorably upon new members who have not proven their ability to save, either in cash or in kind.

Credit

Credit is a loan which commonly needs to be repaid with interest within a specified period of time. Credit can be provided in different ways, including via self-help groups, credit and savings cooperatives, microfinance institutions and commercial banks.

Grants

Grants may be in cash or in kind, such as tools and equipment, animals or agricultural implements. They are often available from government programmes, nongovernmental organizations, foundations, local associations or schemes set up to help vulnerable groups.

Insurance

There are different types of insurance, such as crop, life or health insurance. Providers of financial services sometimes provide insurance as a complement to credit. Some also offer disability insurance, which is an important preventive mechanism to help avoid economic hardship related to possible disablement.

Money transfer systems

For those who cannot work or depend on support from others outside their immediate community, access to an efficient and easy-to-use money transfer system can be important. In some countries, an informal system not linked to banks may be used. Access to money transfer systems may be needed to start a business.

4.5.3 Activities to undertake to improve access to financial services amongst persons with disabilities.

1. Promote the habit of saving

Savings play a vital role in building sustainable livelihoods, particularly for individuals who are poor or vulnerable. Without first cultivating the discipline of saving, it is unwise to encourage someone to take on debt. The practice of saving instills essential habits such as self-discipline, delayed gratification, and planning for future needs that are critical for managing and repaying loans responsibly.

Moreover, accumulating assets through consistent saving strengthens a person's financial stability and enhances their eligibility for mainstream microfinance services. With a solid savings foundation, individuals are better equipped to utilize microcredit effectively and avoid falling into cycles of debt.

CBID Programmes can motivate and teach persons with disabilities and their families to save by:

- o Facilitating the organization of savings groups among persons with disabilities;
- o Helping individuals to open bank accounts in reliable institutions;
- o Helping persons with disabilities to become members of self-help and similar types of savings groups;
- o Building the capacity of group members in administration and financial management

2. Help to confront self-exclusion

Repeated experiences of exclusion and rejection as well as overprotection during childhood can result in a lack of self-esteem or self-confidence. This can easily lead to self-exclusion from services like microcredit. Self-exclusion can also be manifested by the expectation of charity among some persons with disabilities and their families.

The barriers of self-exclusion can be difficult to overcome but, unless they confront this challenge, persons with disabilities may not benefit from other types of interventions. Confronting self-exclusion is first and foremost the responsibility of the individual with a disability and his/her family. Organizations of and for persons with disabilities and CBID Programmes have an important role to play in helping them to fulfil this responsibility through careful and sensitive advice and counselling.

3. Identify role-models

Persons with disabilities need positive role-models to inspire them to take up the challenge of self-employment. Successful entrepreneurs with disabilities exist in many communities; if they are included within the network of a CBID programme they will not only inspire other persons with disabilities, but also change the attitudes of society in general and the microfinance community in particular.

Facilitator note: Possibly identify positive role model before the session

4. Facilitate access to mainstream financial services

Persons with disabilities should have access to financial services on equal terms with nondisabled people. Financial service providers tend to exclude persons with disabilities because of physical or cultural barriers or negative attitudes among their staff. CBID Programmes can facilitate access by:

- o Advocating for equal access for persons with disabilities to mainstream financial services;
- o Identifying financial service providers and presenting entrepreneurs with disabilities as potential clients;
- o Helping financial service providers to be disability-sensitive, so that they can expand their client base to include persons with disabilities;
- o Promoting the idea of hiring qualified employees with disabilities to financial service providers;
- o Encouraging financial service providers to keep the needs of clients with disabilities in mind – staff trained to be more diversity-sensitive, plus appropriate environmental arrangements, are welcomed by all clients;
- o Educating persons with disabilities on how to approach a financial service provider and what to expect when it comes to services and obligations;
- o Helping women with disabilities to access financial services; women with disabilities are often heads of household, responsible for the care of children as well as elderly parents, and need to have control over and manage their own finances to meet family needs better

4.5.4 Role-play (15 min)

Divide the learners into **4 groups** and ask them to prepare for the following role-play on confronting self-exclusion:

Ocak's bike repair shop

Ocak, in Uganda, had polio as a child, which left him with severe mobility problems and sapped his self-confidence. He did not think he would be able to support a family, but was trained by a CBR programme in bike repairing. A training placement was negotiated with an experienced cycle repairer. After three months, Ocak had not only acquired the necessary skills but had also managed to save enough cash to buy a basic set of hand tools. Ocak is now doing very good business under a tree, has a constant queue of customers and has taken on four trainees himself (two of whom have disabilities). He has built up a stock of spare parts. He plans to open an account with a microfinance institution, and ultimately hopes to get a loan to construct a permanent workshop.

- o What specific belief did Ocak hold about himself and his future before the CBR program, and how did this belief reflect an internal barrier to his own participation?
- o How did the CBR program's approach—specifically the practical training and the mentorship—directly challenge Ocak's self-limiting mindset and help him rebuild his self-confidence?
- o By becoming a successful business owner and a trainer for others, including two with disabilities, how has Ocak not only overcome his own self-exclusion but also become a catalyst for the inclusion of others?
- o Beyond a lack of skills or resources, what are some of the social and psychological factors that lead persons with disabilities to self-exclude from economic and social life?

Topic 4.6: Social Protection

Methodology:	Flip Charts, Marker pen, Masking tape, Group discussions
Time:	30 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: What does social protection mean to you? (5 min) 2. In groups of five, participants to discuss on activities to enhance and improve social protection amongst persons with disabilities and make presentations (10 min) 3. In plenary, participants to discuss the challenges experienced in providing social protection (10 min) 4. Facilitators to wrap up the session and read out the key messages (5 minutes)	

4.6.1 Definition

Social protection refers to policies and actions including legislative measures that enhance the capacity and opportunities for the poor and vulnerable groups to improve and sustain their livelihood and welfare; that enable income earners and their dependents to maintain a reasonable level of income through decent work and that ensures access to affordable health care, social security and social assistance.

The groups include orphans and vulnerable children (OVCs), older persons, persons with disabilities, widows and families in street situations.

Social protection measures are intended to provide a safety net to protect people against poverty and loss or lack of income through illness, disability or old age. The overall objective of social protection is to ensure that all Kenyans live in dignity and explore their human capabilities for their own socio-economic development.

Role of CBID

The role of CBID is to facilitate the access of persons with disabilities to mainstream or specific social benefits and promote the provision of social protection measures.

4.6.2 Activities to enhance and improve social protection amongst persons with disabilities

In the following suggested activities, CBID Programmes should work in partnership with persons with disabilities and their representative organizations:-

Ensure that persons with disabilities are included under existing provisions

Social protection is an integral element of a comprehensive approach to disability. CBID programmes need to work in partnership with organizations of and for persons with disabilities to ensure that all persons with disabilities are covered by existing social protection measures, whether these are provided through a social security system, social assistance programme or an informal scheme.

CBID programmes need to be aware of programmes and services which are in place to ensure that families are adequately provided with food, water and work to ensure that the specific requirements of persons with different types of disabilities are adequately taken into account. This may be done in partnership with organizations of and for persons with disabilities. Toilets are a big problem in many parts of the developing world, where many people do not have a toilet in their homes and need to use the natural environment instead. It is difficult for persons with disabilities, especially women with disabilities, to do this. CBID programmes need to work with local authorities to facilitate toilet facilities along with water connections.

Ensure housing and accessibility

The cost of renting suitable accommodation can be higher for persons with disabilities than for others, (for example, they may need a ground floor flat), adding to the daily living costs of people whose income is already limited. Discrimination in housing provision is often encountered by persons with disabilities. CBID Programmes need to be aware of this problem and act as advocates with the relevant authorities to ensure equality of opportunity and treatment for people with disabilities and their families. Accessible accommodation may also become a problem when persons with disabilities leave home to attend training, seek employment or start a small business in another location. Persons with disabilities may not be able to use the available accommodation because it is inaccessible. CBID Programmes need to familiarize themselves with the accessibility requirements of persons with disabilities and work closely with relevant public sector and private agencies to facilitate the finding of suitable housing. Where public housing is being planned, steps should be taken to ensure that the housing is fully accessible, affordable and developed in a way that will contribute to the full participation of persons with disabilities in their communities.

Ensure access to healthcare and assistive devices

Persons with disabilities need access to health-care services on an equal basis with persons who are not disabled. Some persons with disabilities will need health care specific to their health condition/impairment (e.g. drugs for epilepsy) or assistive devices. CBID Programmes need to learn where health-care services and assistive devices are available in the local area and pass on this information to persons with disabilities and their families. In some cases, the CBID programme will directly arrange or facilitate the provision of suitable care or assistive devices.

Ensure personal assistance when required

Some persons with disabilities may require personal assistance with daily care and support. Personal assistance can be provided in communities by volunteers, often family members trained in these roles. Where organizations of and for persons with disabilities and Public Benefit Organizations (PBOs) do not provide personal assistance training, CBID personnel have a central role to play in organizing such training. Fundamental to the provision of personal assistance is the recognition that the person with a disability should be able to choose his/her assistant (see social component: Personal assistance)

Ensure disability-related costs are met where necessary

In some countries, governments provide allowances to cover the disability-related costs of persons with disabilities that are additional to their everyday costs of living. These additional costs may be related to transport, accessible housing, assistive devices or adaptations to everyday articles and equipment. They also include the cost of loss of earnings by volunteer carers, who are mainly women. These costs must be

met if persons with disabilities are to participate fully in family and community life, but they are particularly difficult to meet when persons with disabilities and their families are already living in poverty.

Where such provisions are in place, the CBID programme needs to know about these, make the information available to persons with disabilities and support them in taking advantage of them. Having access to this type of support may make the difference between exclusion and inclusion, and thus between partial and full participation in the community and society.

4.6.3 Challenges of providing social protection

Whatever form of social protection is considered, there is an ongoing debate about who to protect and how. Who is disabled and who should be eligible to receive special social assistance? What is the difference between rights and charity in this context? Is social protection compatible with the ideal of non-dependency?

CBID Programmes have a responsibility to engage with the issue and influence outcomes; along with organizations of and for persons with disabilities they need to act as catalysts, informants, advocates and guides. If they do not, persons with disabilities risk being left out of social protection schemes designed to combat extreme poverty.

KEY MESSAGES

Children with disabilities should also be encouraged to learn, participate and make a contribution in the family.

There are no jobs exclusively “best suited” for persons with disability; rather, like all job seekers, their suitability for employment depends on their unique interests, goals, skills, abilities, and education.

MODULE 5 : SOCIAL COMPONENT

Module description

This module will cover definition of social roles, importance of personal assistance, relationship, marriage & family, culture & arts, recreation, leisure & sports and justice

Module Outcomes:

By the end of the session the learners will be able to: -

- o Define social roles
- o Describe the importance of social roles
- o Identify and address the barriers to community participation.

Module Topics

SN	Topic Names	Duration (Minutes)
5.1	Define social roles	30
5.2	Personal assistance	60
5.3	Relationship marriage and family	60
5.4	Culture and arts	60
5.5	Recreation, leisure and sports	60
5.6	Justice	60
	Total	330

TOPIC 5.1: INTRODUCTION TO SOCIAL ROLES

Methodology:	Flip Charts, Marker pen, LCD, Masking tape
Time:	30 minutes
Facilitator Instructions:	
1. Begin with an icebreaker question: "What are social roles?" (5 min). 2. Divide learners into small groups (5–8 members). Ask each group to share examples of social roles and their importance. (10 min). 3. Discuss the barriers to community participation (10 min) 4. Summarize key points (5 min).	

Facilitator Notes

Being actively included in the social life of one's family and community is important for personal development. The opportunity to participate in social activities has a strong impact on a person's identity, self-esteem, quality of life, and ultimately his/her social status. Because people with disabilities face many barriers in society, they often have fewer opportunities to participate in social activities.

Topic 5.1.1: Definition of social roles

Desirable outcomes

- o Persons with disabilities are valued as members of their families and have a variety of social roles and responsibilities.
- o Persons with disabilities and their families are encouraged and supported to contribute their skills and resources to the development of their communities.
- o Communities recognize that persons with disabilities are valued members, and can make positive contributions to the community.
- o Barriers that exclude persons with disabilities and their families from participating in social roles and activities are challenged and addressed.
- o Local government authorities respond to the needs of persons with disabilities and their family members and provide effective social support and services where required.

Notes to facilitators

- **Step 1;** Facilitator introduces the topic and gives the objectives. (10 mins)
- **Step 2;** Facilitator conducts a plenary session on the participants' understanding of social roles. (20 mins)
- **Step 3;** Facilitator asks participants to brainstorm on the barriers to community participation. (20 mins)
- **Step 4;** Facilitator asks the participants to role play on social roles (20 mins)

Social roles are the positions people hold in society which are associated with certain responsibilities and activities.

Goal for CBID

People with disabilities have meaningful social roles and responsibilities in their families and communities and are treated as equal members of society.

The role of CBID

The role of the CBID is to work with all relevant stakeholders to ensure the full participation of people with disabilities in the social lives of their families and communities. CBID programs can provide support and assistance to people with disabilities to enable them to access social opportunities and can challenge stigma and discrimination to bring about positive social change.

Legislative frameworks. (Add)

The social inclusion of persons with disabilities is protected under several key legal and policy frameworks.

- The UN Convention on the Rights of Persons with Disabilities (UNCRPD) affirms equal participation in community and social life through Articles 19, 23, 30, and 13, which guarantee the right to independent living, family life, participation in culture, recreation and sports, and access to justice.
- The Constitution (2010) upholds equality, non-discrimination, family protection, and access to justice (Articles 27, 45, 48, and 54).
- The Persons with Disabilities Act (2025), National Social Protection Policy (2011), and Sports Act (2013) reinforce inclusive participation, personal assistance, and access to leisure, culture, and justice systems. Within CBID, these laws guide community efforts to promote equal social roles, independent living, inclusive families, accessible recreation, and full participation of persons with disabilities in cultural and justice systems.

- SDG 10 – calls for reducing inequalities within and among societies.

Why social roles are important

- Adds value to life
- Foster changes in social attitude
- Helps maintain social order

Barriers to community participation

A wide range of barriers may restrict the community participation of people with disabilities. For example:

- o Persons with disabilities may have poor self-esteem and think that they do not deserve or have the ability to take part in activities and events;
- o Family members may feel that having a member with a disability brings shame, so they do not encourage or allow this person's community participation;
- o Community members may have irrational thoughts and beliefs about disabilities, e.g. That holy places are defiled by a person with a disability, that persons with disabilities are cursed and need cleansing, that persons with disabilities have supernatural or evil powers;
- o Physical barriers to community participation include inaccessible transport and buildings, e.g., community centers, sporting venues, and cinemas
- o Communication barriers as a result limited alternative means of communication, e.g. sign language interpreters, materials in braille, digital platforms etc.

TOPIC 5.2: PERSONAL ASSISTANCE

Module outcome

By the end of the session the learner should be able to: -

- To define the personal assistance concept
- Sensitize on the disability legislative frameworks.
- To understand the importance and challenges of personal assistance

Instructions to the facilitator

- **Step 1;** The Facilitator introduces the topic and gives the objectives.
- **Step 2;** The Facilitator asks participants to brainstorm on personal assistance tasks, informal assistance and formal assistance.
- **Step 3;** The Facilitator provides insight on personal assistance tasks, informal assistance and formal assistance.
- **Step 4;** The Facilitator divides the participants into small groups to list and discuss challenges of personal assistance.
- **Step 5;** Group presentation in plenary.
- **Step 6;** Facilitator provides insight on challenges of personal assistance.

Introduction

Some persons with disabilities require personal assistance to enable them to participate in family and community life. In low-income countries where there are limited social services and benefits, families are often the main or only means of this support. While formal options are often very limited, this element shows how CBID programs can utilize community resources to ensure persons with disabilities and their families are able to access a range of different options for personal assistance which are appropriate to their individual needs and preferences.

5.2.1 Definitions of Concepts

Personal Assistance

The direct, one-on-one support a person with disabilities requires to achieve the same range of self-determination, opportunities, and activities as a nondisabled person.

Personal assistance tasks

Personal assistance is not only about supporting people with intimate personal tasks. It may also involve supporting them with a variety of tasks in different environments in homes (assistance with parenting and caregiver tasks), schools, workplaces, community settings (e.g. traveling, shopping, banking), and emergency respite services. Regardless of whether personal assistance is provided through informal or formal means, it is important that it is of a quality and quantity that appropriately supports the person with disabilities to fully participate with dignity in family and community life

Informal assistance

Worldwide, most support to persons with disabilities takes the form of informal assistance. Family members, friends, neighbors and/or volunteers may provide this informal assistance.

Formal assistance

Personal assistance may also be provided on a formal basis. Many different types of formal support services are offered in high-income countries, and increasingly also in low-income countries (3). These services may be provided by governmental and nongovernmental organizations and the private sector. As more budgets become decentralized, funds for support services may become available through local authorities, such as ministries of social welfare. Allowances, such as disability pensions, guardianship awards or caregiver allowances, may be available to fund personal assistance.

Goal of CBID

People with disabilities are able to access personal assistance which meets their needs and enables them to lead active and fulfilling lives.

The role of CBID

The role of CBID programs is to support people with disabilities to access and actively manage the personal assistance necessary to live their lives with self-determination and dignity.

Desirable outcomes

- o CBID programs and organizations of and for persons with disabilities work together to ensure personal assistance options of an appropriate quality and standard are available in the local community.
- o Persons with disabilities have individual support plans in place.
- o Persons with disabilities have access to training to enable them to manage their personal assistance needs.
- o Training is available for both informal and formal personal assistants.
- o Support is available for families who provide personal assistance on an informal basis.
- o Mechanisms are in place in the family and community to respond to crisis situations and prevent people with disabilities being sent into institutional care.
- o Local communities support and provide community-based personal assistance options for people with disabilities

Challenges

When trying to ensure that persons with disabilities are able to access personal assistance options in low-income countries, CBID programs may face challenges, such as:

- Little awareness about and demand for personal assistance by people with disabilities, their families and organizations for and of persons with disability.
- Scarcity of programs that provide training and support for personal assistants.
- No social protection policies and programs in place to support personal assistance, e.g. Disability allowances, grants or pensions (only in a few low-income countries).

Role play;

Set- up: Split the participants up into pairs and assign each pair from a predetermined list of personal challenges. One participant will portray a person dealing with this difficulty, while the other will be their friend, neighbors, social worker etc.

Role-playing: one participant listens sympathetically as the other participant expresses themselves highlighting their problems and feelings. After 10 mins the participants switch roles. During the session open-ended questions should be the main focus of the session in order to promote greater introspection and comprehension.

TOPIC 5.3: RELATIONSHIPS, MARRIAGE AND FAMILY

Module outcome

By the end of the session the learner should be able to: -

- Define the concepts on Relationships, marriage and family
- Explore the importance of relationships and identify ways of promoting healthy marriages and families
- Understand the Disability legislative frameworks on the topic
- Understand the social networks and supports that are in place to protect persons with disabilities from violence

Facilitator Instructions:

1. Begin with an icebreaker question: "What do you understand by marriage and family? (5 min).
2. Divide learners into small groups (5–8 members). Ask each group to share the importance of healthy relationships. (10 min).
3. Discuss the social networks and supports that are in place to protect persons with disabilities from violence (10 min)
4. Summarize key points (5 min).

5.3.1 Introduction

Relationships are as important for persons with disabilities as for everyone else. This element looks at the ways in which CBID programs can support persons with disabilities to enjoy a variety of social roles and responsibilities associated with relationships. By working closely with a range of stakeholders in the community, CBID programs can help to increase disability awareness, challenge negative family and community attitudes, and prevent and address violence against persons with disabilities.

Family responses to disability

Every family responds differently to disability. One family may find it difficult to accept the birth of a disabled child due to fear and may need more information about what the future holds, while some may celebrate the birth of their child.

Most parents and caregivers love their children regardless of their unique circumstances, and would like

to do what is best for their children, but parents and caregivers' actions/behavior are not always right especially towards a child with disability, a parent/caregiver's negative words/actions can be harmful to the child, they can continue to bring stress and pain even when they become adults. Families can be effective advocates and powerful agents of change to ensure the inclusion of persons with disabilities. They can have a positive influence on the expectations of family members with disabilities and also influence the attitudes of the community.

5.3.2 Definition of concepts

Relationships: a state involving mutual dealings between people or parties or fulfilling personal relationships are important to everyone and are essential for personal growth and development. Stimulating, lasting and satisfying relationships with family, friends and partners are a high priority for most people, including people with disabilities.

Marriage: the legally or formally recognized union of two people as partners in a personal relationship.

Family: a group of one or two parents and their child or children living together as a unit. The word "family" can mean different things to different people. There are different types of families, all are equally valuable and what is important is that they provide a sense of belonging, love, care, connection, support and meet basic needs (food, shelter, clothing, health and education). Families can also provide an environment for learning and development and provide safety and security for children and vulnerable family members including people with disabilities. All children including those with disability, who grow up in protective and caring families are more likely to become caring and responsible adults.

Disability legislative frameworks

Families are diverse and influenced by a range of factors including cultural, traditional and religious practices. Families may be classified as nuclear, extended, single-parent, child-headed, foster, or adoptive. It is important to recognize this diversity and also recognize that persons with disabilities have a right to establish their own families. Article 27, from the Constitution of Kenya 2010 entitles every person to equality before the law and prohibits direct or indirect discrimination on any ground, including disability. Article 28, guarantees the right to human dignity and the right to have that dignity respected and protected.

Marriage and Positive Parenting

It is often assumed that persons with disabilities will automatically have children with disabilities. There is also a common belief that persons with disabilities will not be able to care for and support their children independently. In extreme cases, persons with disabilities, particularly adolescent girls and women with intellectual disability, have been sterilized without their knowledge or consent. Persons with Disabilities ACT no.6 of 2025, every person with disability who is marriageable age has the right to marry and form a family,

Sexuality

Sexuality is an important part of health and well-being. However, in many societies discussion of the subject is sensitive or even a taboo, particularly if it relates to disability, around which myths and misconceptions may go unchallenged. For example, persons with disabilities are often viewed as being asexual, infertile, or as having overly high sex drives; these views may not only be held by community members but also by medical professionals and in some cases persons with disabilities themselves. It is important to be aware that persons with disabilities have sexual needs, as does everyone else.

5.3.3 How CBID can provide support for parents and caregivers

To enable parents and caregivers to make good parenting decisions, access to information and support is needed. CBID programs should consider both parents with disabilities and parents of persons with

disabilities. They can provide support by:

Training parents and caregivers of children with disability on positive parenting;

- o Identifying local services that can provide support to parents, e.g. In sexual and reproductive health, maternal and child health, and family support;
- o Advocating with organizations for persons with disability and other organizations for the inclusion of parents with disabilities, and parents of people with disabilities, in mainstream services and programs;
- o Developing and supporting referral systems to facilitate access for persons with disabilities, especially women and adolescents, to services and programs;
- o Working with service providers to distribute accurate information on sexual and reproductive health in accessible formats through CBID networks

How CBID can work with families

Sometimes families overprotect their members with disabilities by sheltering them in the home and preventing them from socializing in the community, thus limiting their opportunities to develop relationships with others or to develop various skills and abilities. CBID programs can work with families to:

- o Provide information and support to address their concerns regarding their members with disabilities, as well as for their own position and status within the community;
- o Help families to recognize the negative consequences of overprotection;
- o Encourage family members to become advocates for changing negative attitudes within the community;
- o Support persons with disabilities to effectively communicate and self-advocate for their own needs and wants.

5.3.4 Concept and types of Violence

Violence can occur in every part of society, within families and institutions, and in schools, workplaces, and the community; many different people are responsible. Persons with disabilities are disproportionately susceptible to violence, particularly because of stigma, negative traditional beliefs and ignorance; they are at increased risk of becoming victims of physical, sexual, psychological and emotional abuse, neglect, and financial exploitation.

Help to prevent violence through CBID programs

Violence of any type can be a challenging issue for CBID personnel. As CBID programs work across different environments (e.g. homes, schools, workplaces, communities) they are in a good position to ensure that strong social networks and support are in place to protect people with disabilities from violence (see also Justice). CBID programs can:

- o Build the capacity of CBID staff to recognize the signs and symptoms of violence, ensuring they know where to access appropriate legal advice and support for persons with disabilities;
- o Raise awareness within communities about violence, disability and actions that can be taken to protect persons with disabilities.
- o Establish links with relevant stakeholders (e.g. Families, Organizations of and for persons with disabilities, health-care/education personnel, law enforcement officers, community leaders, local authorities), and enlighten them on their role in protecting persons with disabilities from violence;
- o Develop procedures with stakeholders which enable persons with disabilities to report episodes of violence in confidence.

- o Provide information about violence to persons with disabilities and ensure they know how they can report episodes of violence confidentially.
- o Ensure persons with disabilities have opportunities to participate in community life to increase their self-esteem, confidence, and develop social networks which will help protect them from violence.
- o Support persons with disabilities who have experienced episodes of violence by providing psycho-social support assisting them to access health-care services, and assisting them to develop solutions and take action.
- o Ensure that programmed and organizational policies are in place to check that CBR staff and volunteers have no criminal history of violence.

5.3.5 Challenge and address stigma, prejudice and discrimination

Negative attitudes, perceptions and practices towards persons with disabilities are present in many communities. CBID programs can challenge and address these by:

- Working with the media to promote positive images and role models of persons with disabilities;
- Supporting disability awareness training for health professionals to ensure that sexual and reproductive health services, e.g. Family planning, are accessible to persons with disabilities;
- Working with leaders in the community, e.g. religious leaders, to encourage them to create awareness about disability, challenge stigma and discrimination, and create opportunities for community discussions about sensitive issues

Goal of CBID

Persons with disabilities fully realize their positions and roles within their families and communities.

The role of CBID

The role of CBID is to support persons with disabilities to have fulfilling relationships with their families and communities.

Desirable outcomes

- o Community members are aware and acknowledge that persons with disabilities can have meaningful relationships, marry and have children.
- o Parents with disabilities and parents of persons with disabilities have access to appropriate services and programs to support them in their parenting roles.
- o Families encourage and support their members with disabilities to socialize and develop relationships outside the home.
- o Persons with disabilities are protected against violence, and all relevant stakeholders work together to address the issue.
- o Persons with disabilities who have limited social networks are well supported in their communities.

Facilitator notes

- **Step 1;** facilitator introduces the topic and gives the objectives.
- **Step 2;** Facilitator divides the participants into small groups to list and discuss how CBID programs can provide support for parents, how it can work with families to promote independence, what is Violence and how to prevent Violence using CBID Programmes.
- **Step 3;** Group presentation in plenary
- **Step 4;** Facilitator provides insight on how CBID programs can provide support for parents, how it can work with families to promote independence, what is Violence and how to prevent Violence

using CBID Programmes.

- **Step 5;** The facilitator provides input on how CBID programs can challenge and address stigma, prejudice and discrimination.
- **Step 6:** Role play activity:

Set-up: in groups of 5 participants to play out a range of situations in which prejudice could affect actions or choices (e.g., disability, single parenthood, divorced, married etc.). A character participating in these choices is assigned to each participant according to their demographic characteristics (race, gender, disability and age).

Role-playing groups simulate scenarios where prejudices could manifest, like living situations, marriage. Participants must be conscious of how bias affects interactions and results while navigating their roles.

TOPIC 5.4: CULTURE AND ARTS

Module outcome

By the end of the session the learner should be able to: -

- Define concepts on Culture and Arts
- Understand the disability legislative framework

Identify the benefits of preservation and participation in Culture and Arts

Facilitator Notes

- **Step 1;** Facilitator introduces the topic and gives the objectives.
- **Step 2;** Facilitator divides the participants into small groups to list and discuss benefits of participation in Culture and Arts and activities for awareness creation through Culture and Arts.
- **Step 3;** Group presentation in plenary.
- **Step 4;** The facilitator provides input on the benefits of participation in Culture and Arts and activities that can be used for awareness creation through Culture and Arts.
- **Step 5;** The facilitator asks participants to brainstorm on how CBID programs can support families to participate, Encourage Persons with disability to participate, how it can work with mainstream Organizations and groups and how it can work with spiritual leaders and religious Leaders.
- **Step 6;** The facilitator provides input on how programs can support families to participate, Encourage Persons with disability to participate, how it can work with mainstream Organizations and groups and how it can work with spiritual leaders and religious Leaders.
- **Step 7;** Role plays activity.

5.4.1 Introduction

The word “culture” has many different meanings. In this element however, it is used to refer to the way of life of a group of people. Culture therefore includes many things, such as dress, food, language, values and beliefs, religion, rituals and practices. The arts are also closely associated with culture and include painting, music, dance, literature, film, photography.

Convention on the rights for persons with disability Article 30; participation in cultural life, recreation, leisure and sports States parties recognize the right of persons with disability to take part on an equal basis with others in cultural life, and shall take all appropriate measures to ensure that persons with disability:

- Enjoy access to cultural materials in accessible formats;
- Enjoy access to television programs, films, theatre and other cultural activities, in accessible formats.

- Enjoy access to places for cultural performance or services, such as theatres, museums, cinemas, libraries and tourism services, and as far as possible, enjoy access to monuments and sites of national cultural importance.

Benefits of participation in culture and arts

- Participation in cultural and arts activities not only entertains but also teaches individuals about who they are.
- The process of participating can be empowering for individuals; it can help them find their own voice and be heard by others.
- It also has many health benefits, for example, enhances cognitive, motor, mental health and communication skills.
- For some persons with disabilities, cultural and arts-based activities may be the only means through which they can fully represent themselves on their own terms and on an equal basis with others.
- The presence of cultural and arts activities in the community contributes significantly to the well-being of its members. These activities can develop and strengthen relationships within and across communities, build capacity within the community, and promote community development and regeneration.
- The involvement of persons with disabilities in the cultural and artistic aspects of their communities is often a good indicator that community members have positive attitudes towards disability.

Activities for awareness creation through Culture and Arts

Disability advocacy and awareness campaigns are often more effective when based on locally relevant cultural and artistic media. Therefore, CBID programs should consider using these media to achieve social change for disability. Programmes can:

- Work with artists and organize drama, arts and music events to challenge disability, stigma and discrimination that may be present in the community.
- Explore the use of comedy, cartoons and popular art as a light and non-threatening way to address taboo subjects.
- Support the positive portrayal of persons with disabilities and disability issues through cultural and artistic media, e.g. Drama, movies and theatre.

5.4.2 How CBID programs can support families to participate

It is important to remember that many families of persons with disabilities may have lost the confidence to attend cultural events and venues through fear of stigma and discrimination. They may feel embarrassed, ashamed or unwelcome at weddings, religious services, restaurants, and cinemas. It is important that CBID programs work with families and provide support by:

- Listening to family members and encouraging them to express, recognize and challenge their fears.
- Putting families in touch with others who share similar experiences and concerns.
- Facilitating links to local organizations of and for persons with disabilities to examine any misconceptions families may have and increase their confidence, expectations and aspirations.

How CBID programs can encourage persons with disability to participate

CBID programs can encourage persons with disabilities to participate by:

- o Connecting them with groups and organizations of persons with disabilities where they may

find people who share similar interests in culture and the arts, and where they may develop the confidence to participate in a variety of activities.

- o Identifying artists with disabilities who have achieved recognition for their art and involving them in developing and implementing programmed activities.
- o Working with stakeholders to develop specific disability arts projects.
- o Supporting and promoting examples of persons with disabilities who have excelled and developed new art forms, e.g. Silent theatre, and art forms which have been successfully adapted to reach wider audiences.
- o Promoting and supporting dance, drama and music as complementary types of therapy for persons with disabilities.

Role of CBID programmed in Working with mainstream organizations and groups

CBID programs have an important role to play in working with mainstream organizations and groups to develop their skills and confidence to include persons with disabilities in their cultural and artistic programs and activities. CBID programs can:

- o Work with stakeholders to ensure that reasonable accommodation is made to enable inclusion of persons with disabilities, such as modifying buildings to ensure physical access (e.g. Community halls, places of worship, cinemas, tourist attractions), or producing information and promotional materials in accessible formats.
- o Develop alliances with women's groups to increase the inclusion of women with disabilities in mainstream cultural programs.
- o Encourage inclusive art education in early childhood and school settings so that children with disabilities have opportunities to appreciate and participate in cultural and artistic activities from a young age.
- o Support persons with disabilities and disabled organizations of and for persons with disabilities to develop and run diversity training with staff from a range of cultural and arts programs to raise awareness about disability and inclusion.
- o Encourage apprenticeship and employment opportunities within cultural and arts programs to ensure that persons with disabilities actively participate in the management and administration of these programmed.

How CBID Program me can work with Faith based organizations

Spiritual and religious leaders can be powerful advocates for social change; however, they can also promote negative attitudes. So, it is important for CBID programs to work with religious leaders and members of all faiths within the community to promote the inclusion of people with disabilities in their activities. CBID programs can:

- o Sensitize leaders about disability and the importance of including persons with disabilities in religious and spiritual activities.
- o Encourage leaders to challenge discriminatory or harmful practices towards persons with disabilities and their families in the community.
- o Ensure persons with disabilities have access to personal assistance to enable them to attend religious/spiritual services and programs.
- o Provide advice and assistance to leaders about making information accessible, e.g. making prayers, songs, chanting, and sermons accessible with signed translation, and making religious texts available in large print, audio and braille.
- o Work with leaders to ensure that places of worship are physically accessible and that religious practices are modified to accommodate persons with disabilities.

Goal of CBID

Persons with disabilities contribute to and participate in the cultural and artistic lives of their families and communities.

The role of CBID

The role of CBID programs is to work with relevant stakeholders to enable persons with disabilities to enjoy and participate in cultural and arts activities.

Desirable outcomes

- o Stigma and discrimination towards disability is challenged and addressed through culture and art.
- o Parents and caregivers understand the risks that some societal beliefs and practices have on children with disabilities and take practical steps to promote positive beliefs and norms.
- o Persons with disabilities and their family members participate in a range of cultural and arts events and activities.
- o Mainstream organizations and groups support the inclusion of persons with disabilities in their cultural and artistic programs and activities.
- o Persons with disabilities are able to access mainstream cultural/arts media and venues.
- o Spiritual and religious leaders and groups include persons with disabilities in their activities.

TOPIC 5.5: RECREATION, LEISURE AND SPORTS

Module outcome

By the end of the session the learner should be able to: -

- o Define concepts on Recreation, Leisure and sports.
- o Sensitize the disability legislation frameworks.
- o To identify the opportunities and benefits of Recreation, Leisure and sports on persons with disability.

Notes to facilitator

- **Step 1;** Facilitator introduces the topic and gives the objectives.
- **Step 2;** Facilitator divides the participants into small groups to list and discuss benefits of Recreation, Leisure and sports in relation to CBR.
- **Step 3;** Group presentation in plenary.
- **Step 4;** The Facilitator provides input on the benefits of Recreation, Leisure and Sports in relation to CBR.
- **Step 5;** Role plays activity.

Introduction

Recreational, leisure and sporting activities are important for health and well-being and for strengthening the cohesion of the community. This element looks at the benefits these activities can have to persons with disabilities and provides practical suggestions about how CBR programs can work with a range of stakeholders to increase opportunities for the inclusion and participation of persons with disabilities. The value of working directly with stakeholders to plan and develop Programmes and activities that are appropriate and responsive to the local community is highlighted.

Definitions In this element:

Recreation

Refers to all those activities that people choose to do to refresh their bodies and minds and make their leisure time more interesting and enjoyable. Examples of recreation activities are walking, swimming, meditation, reading, playing games and dancing.

Leisure

Refers to the free time that people can spend away from their everyday responsibilities (e.g. work and domestic tasks) to rest, relax and enjoy life. It is during leisure time that people participate in recreation and sporting activities.

Sports

Refers to any type of organized physical activity, e.g. soccer, rugby, football, basketball and athletics.

Benefits of recreation, leisure and sports

1. Improving the health and well-being of individuals,
2. Contributing to the empowerment of individuals, and
3. Promoting the development of inclusive communities' activities.

Goal of CBID

Persons with disabilities participate both actively and as spectators in recreational, leisure and sporting activities on an equal basis with others.

The role of CBID

The role of CBID Programmes is to promote increased participation of persons with disabilities in recreation, leisure and sports activities; and provide support to mainstream organizations and Programmes to enable them to strengthen their capacity by offering appropriate and accessible recreation, leisure and sports activities.

Desirable outcomes

- o Persons with disabilities participate in recreation, leisure and sports Programmes available in the local community.
- o Local, national and international authorities and associations include people with disabilities in their recreational, leisure and sports Programmes.
- o Families, teachers and community members recognize and actively promote the right and ability of people with disabilities to take part in recreation, leisure and sports activities.
- o Persons with and without disabilities are both involved in recreation, leisure and sports activities.
- o persons with disabilities are able to access recreation, leisure and sports venues.
- o Equipment used for recreation, leisure and sports is adapted where needed to accommodate the needs of people with disabilities.
- o Recreational, leisure and sports Programmes and activities are developed specifically for persons with disabilities where required.

The benefits of participation

Participation in recreation and sports activities can have many benefits for both the individual and community. These include:

- o Health promotion and disease prevention – recreation and sports activities are an enjoyable and effective way to improve health and well-being; they can relieve stress, increase fitness, improve physical and mental health, and prevent the development of chronic diseases, such as heart disease;
- o Skills development – physical and social skills are some of the many skills that can be developed through participation in recreation and sports activities;
- o Awareness raising, reduction of stigma and social inclusion – recreation and sports activities are a powerful, low-cost means to foster greater inclusion of persons with disabilities; they bring people of all ages and abilities together for enjoyment, and provide persons with disabilities the opportunity to demonstrate their strengths and abilities, and promote a positive image of disability;
- o International peace and development – sport is a universal language that can be used as a powerful tool to promote peace, tolerance and understanding by bringing people together across boundaries, cultures and religions,
- o Empowerment – recreation and sports activities can empower persons with disabilities by positively influencing their self-confidence and self-esteem.

TOPIC 5.6: JUSTICE

Module outcome

By the end of the session the learner should be able to: -

- o Define the concepts in Justice
- o Sensitization on the disability legislation frameworks
- o Sensitize on the avenues for seeking justice
- o Identify barriers to justice to persons with disability

Identify the various alternative dispute resolution mechanisms.

Facilitator Notes

Note to facilitator

- **Step 1;** Facilitator introduces the topic and gives the objectives(10mins).
- **Step 2;** Facilitator asks participants to brainstorm on the barriers to justice and Legal capacity (10mins)
- **Step 3;** Facilitator provides input on the barriers to justice and Legal capacity (10 mins)
- **Step 4;** Facilitator divides the participants into small groups to discuss justice, where to get justice and different types of justice. (20 mins)
- **Step 5;** Group Presentation in Plenary (20mins)

Introduction

All community members may need to access justice at some point in time. At a local level it is important to be aware of existing laws that can be used to ensure persons with disabilities are able to access their rights and entitlements as outlined in the Convention on the Rights of Persons with Disabilities. This element focuses on how CBR Programmes can support persons with disabilities to claim their rights by raising their legal awareness and facilitating their access to a range of legal processes to challenge injustices

5.6.1 Definition of concept

Justice

Justice is the ethical, philosophical idea that people are to be treated impartially, fairly, properly, and reasonably by the law and by arbiters of law, that laws are to ensure that no harm befalls another, and that, where harm is alleged, a remedial action is taken both the accuser and the accused receive a morally right consequence merited by their actions.

Rights holders

Refers to individuals or social groups that have particular entitlements in relation to specific duty-bearers. Persons with disabilities are rights holders, i.e. they have both entitlements and responsibilities. As rights holders they are entitled to, for example, health, education, livelihood opportunities, land, housing, and political participation.

Duty bearers

Those actors who have a particular obligation or responsibility to respect, promote and realize human rights and to abstain from human rights violations. The term is most commonly used to refer to State actors, but non-State actors can also be considered duty bearers. When a person has a right, someone else has a duty to respect, protect and fulfil that right. Duty bearers include both state (government) and non-state (e.g. nongovernmental organizations, religious leaders, parents) actors at national and local levels.

Alternative dispute resolution:

Refers to any methods used to resolve a dispute between parties without resorting to litigation. examples are; mediation, arbitration, conciliation, negotiation, and transaction.

5.6.2 Barriers to justice

Access to justice is often limited to the wealthy, the politically connected, and people living in urban areas. Persons with disabilities, particularly those who live in low-income and middle-income countries, face a number of barriers to accessing justice. These barriers include:

- o Absence of adequate laws and policies to protect persons with disabilities – in many countries there are no specific laws to protect their rights;
- o Physical barriers – persons with disabilities may be unable to access police stations, courts, and other public buildings;
- o Barriers in communication – persons with disabilities may be unable to communicate effectively with people in the justice sector if reasonable accommodations are not in place, e.g. sign language interpreters for people who are deaf;
- o Lack of accessible information – without accessible information persons with disabilities may not be aware of how the justice system works and what their rights and responsibilities are;
- o Economic barriers – the cost of lawyer's fees, court fees, etc. may be too high for persons with disabilities; lack of awareness of needs – police and other officials may not understand the specific needs of people with disabilities in accessing justice and information on how to provide the necessary accommodations

5.6.3 Legal capacity

- o Article 45 of the constitution of Kenya provides for the recognition of family as the basic natural and fundamental unit of Society.
- o Article 48 Access to Justice; the State shall ensure access to justice for all persons and, if any fee is required, it shall be reasonable and shall not impede access to Justice.
- o Article 11 Culture; This constitution recognizes culture as the foundation of the nation and as the cumulative civilization of the Kenyan People and nation.
- o Persons with disability Act 2025 no. 8 Right to marry and form a family

- o Social protection act 2023
- o Children's Act 2022

Raise awareness about rights

Strategies to promote legal awareness can be undertaken by CBID programmed; they can:

- o Work organizations of persons with disabilities to ensure that people with disabilities and their families are aware of their rights;
- o Disseminate information in accessible formats on disability rights and how to access these rights;
- o Participate in awareness raising activities along with organizations of persons with disabilities, human rights organizations and self-help groups;
- o Support organizations of persons with disabilities and human rights organizations to carry out disability training with key sectors and decision-makers at the community and district levels, e.g. Law enforcement and court officers, legal and health professionals, teachers, religious and business leaders.

Promote access to informal mechanisms where appropriate

Formal legal processes may not always be the most appropriate mechanism. Often informal mechanisms can be more effective and are usually quicker, less costly and more accessible for community members. Examples of ways in which CBID Programmes can support access to informal methods of justice include:

- o Working with local schools to encourage them to enroll children with disabilities;
- o Approaching the local community and religious leaders to help resolve family disputes, e.g. A dispute about a couple's right to marry when one or both partners has a disability;
- o Working with farming cooperatives to ensure that farmers with disabilities have access to collective community resources;
- o Working with banks to enable clients with disabilities to manage their own accounts and access credit;
- o Supporting a local doctor to have a sign language interpreter so that persons with hearing impairment members of the community can access and use health-care facilities;
- o Working with tribal or religious leaders and heads of families to support persons with disabilities to claim their inheritance rights.

Support legal action where appropriate

It is important for CBID Programmes to:

- o Build strong relationships and alliances with trusted members of the legal community;
- o Seek legal advice to find out which legislation is appropriate for addressing the different types of discrimination, e.g. Local agreements, national legislation or international conventions or treaties;
- o Respect the decision of the person with disability to seek legal action;
- o Review the risks involved in taking legal action, e.g. Time, costs, and security considerations, particularly where effective legislation and protection mechanisms may not exist;
- o Ensure that people with disabilities and their families are involved in legal processes and are also aware of the risks involved;
- o Develop an awareness of, and work together with, organizations which address legal issues for poorer members of the community (e.g. Community or legal aid centers, lawyers' collectives and/or international human rights and development organizations).

Goal of CBID

Persons with disabilities have access to justice on an equal basis with others to ensure full enjoyment and respect of human rights.

The role of CBID

The role of CBID is to promote awareness of the rights of persons with disabilities and provide support to persons with disabilities and their family members to access justice where they face discrimination and exclusion.

Desirable outcomes

- o CBID Programmes are able to support persons with disabilities to access justice when the need arises.
- o Persons with disabilities are aware of their rights and the options and procedures for accessing justice.
- o Relevant stakeholders in the justice sector are sensitive to the needs of persons with disabilities and do not engage in discriminatory practices.
- o Persons with disabilities are able to access informal mechanisms of justice when their rights are violated.
- o Persons with disabilities are able to access formal mechanisms of justice when their rights are violated.

MODULE 6 : EMPOWERMENT COMPONENT

Module description

This module will cover the concept of empowerment, advocacy and communication, Organizations of and for persons with disabilities, community mobilization, Community Groups and political participation.

Module Outcomes:

By the end of the session the learners will be able to: -

1. Describe key concepts of empowerment
2. State the importance of advocacy and communication
3. Describe the roles and importance of OPDs.
4. Explain community mobilization
5. State the importance of Community groups
6. State the importance of political participation to persons with disabilities.

Module Topics

SN	Topic Names	Duration (Minutes)
6.1	Concepts of empowerment	20
6.2	Importance of advocacy and communication	60
6.3	Roles and importance of OPDs.	60
6.4	Community mobilization	60
6.5	Importance of Community groups	60
6.6	Importance of political participation	60
	Total	320

TOPIC 6.1: INTRODUCTION TO EMPOWERMENT

Methodology:	Flip Charts, Marker pen, LCD, Masking tape
Time:	30 minutes
Facilitator Instructions:	
1. Define empowerment (5 min). 2. Define awareness (5 min). Define participation (5min). 3. Summarize key points, (5 min).	

6.1.1 Definition of terms

Empowerment

Empowerment is the process of helping individuals or groups gain the skills, resources and confidence to have a say and being listened to, self-power, own decision-making, having control or gaining further control, being free, independent, being capable of fighting for one's rights, being recognized and respected as equal citizens with a contribution to make.

Empowerment is a complex process; it is not something that happens immediately, or that can be given to someone. Change must start with communities including community members, stakeholders and persons with disabilities shifting their mindsets and becoming active contributors to achieve empowerment. This shift in thinking is important for overcoming the attitudinal, institutional and physical barriers that may be present in the community. CBID Programmes can facilitate this process by; raising awareness, providing information, capacity building, encouraging participation, which can lead to greater control and decision-making.

Awareness

Awareness is the level of understanding that individuals have of themselves, their situation and the society at large. Disability awareness encompasses knowledge of lived experiences of persons with disabilities, recognizing barriers that hinder inclusion, challenging misconceptions that reinforce discrimination and stigmatization. Awareness must ensure an understanding of the unique strengths, preferences and contributions of all persons with disabilities in the Society. Cross sharing information ensures that people are better equipped to take advantage of opportunities, and understand their rights and obligations of stakeholders including the Government as the duty bearer. Raising awareness assists people to recognize that there are opportunities for change in the families and communities about disability issues and human rights. This can also help to remove barriers for people with disabilities so they have greater freedom for participation and decision-making.

Participation

Participation is the active involvement of community members in planning, decision making and implementation of projects affecting their lives. Persons with disabilities can participate meaningfully and make positive contributions at various levels; example, at a household level they can participate in activities such as caring for family members, herding animals, fetching water, cooking, cleaning; at a community level they can provide peer support to people who have recently experienced disability and participate as members of a group or organization.

Therefore, being a contributor brings social recognition and is important in the empowerment process.

Takeaways from this Section.

- o **Empowerment** is about having a voice, making decisions, being independent, and being respected as equal citizens.
- o It is a **gradual process** that needs both persons with disabilities and communities to shift their thinking and break barriers.
- o **Awareness** helps people understand disability issues, recognize strengths, challenge stigma, and know rights and responsibilities.
- o **Participation** means being actively involved at home and in the community, which builds recognition and strengthens empowerment.

6.1.2 Legal and Policy Linkages

Empowerment underpins all CBID components and is rooted in Article 29 of the CRPD, which ensures participation in decision-making and self-advocacy.

Supporting frameworks include:

- o The Constitution of Kenya (2010) – Articles 54 and 55 promote participation, representation, and leadership of marginalized groups.
- o Persons with Disabilities Act (2025) – establishes various platforms for representation and empowerment.
- o National Gender and Equality Commission Act (2011) – upholds equality and inclusion for all.
- o SDG 16 – promotes inclusive institutions and participatory decision-making.

Empowerment within CBID means building the confidence, leadership, and advocacy capacity of persons with disabilities and their organizations to influence decisions affecting their lives and communities.

6.2: ADVOCACY AND COMMUNICATION

Module outcome

By the end of the session the learner should be able to: -

- o Define self-advocacy
- o State and explain importance of self-advocacy
- o Discuss the support required by persons with disabilities for self-advocacy
- o Define communication
- o Discuss characteristics of effective communication
- o Discuss communication barriers
- o Explain ways of addressing communication barriers

Facilitators notes.

- o **Step 1:** The facilitator defines self-advocacy and communication.
- o **Step 2:** The facilitator outlines the importance of self-advocacy and support required by persons with disabilities.
- o **Step 3:** In small groups of 5 learners, discuss characteristics of effective communication and barriers to effective communication
- o **Step 4:** In plenary groups present their findings and the facilitator clarifies unclear areas
- o **Step 5:** The facilitator outlines ways of addressing communication barriers.

6.2.1 Advocacy.

Self-advocacy is the ability of a person having a voice of their own. It means knowing about their rights and responsibilities, speaking up for their rights, and being able to make choices and decisions about their lives that affect them.

Importance of self-advocacy

- o By learning self-advocacy skills, persons with disabilities can learn to assert their rights.
- o Take control of their lives, and make the best decisions for themselves and their families.
- o Self-Advocacy can be as simple as persons with disabilities making choices about their holistic lifestyles that reinforce social inclusion.
- o It may mean that they speak out publicly about issues which are important to them such as commenting about services that they ought to have received and used.

Support required by persons with disabilities

In many settings persons with disabilities experience stigma and discrimination, and it can be very difficult to voice their opinions, in some situations, they may request or need the support of an advocate. An advocate might be a family member, friend, member of organizations of and for persons with disabilities, an advocacy volunteer from the community.

An advocate may assist a person with a disability to access information, provide support at appointments and meetings, write letters about important issues and speak on their behalf in difficult situations.

Takeaway- Advocacy.

- o **Self-advocacy** means having your own voice: knowing your rights and responsibilities, speaking up, and making your own choices.
- o It helps persons with disabilities **assert their rights, take control of their lives, and make better decisions** for themselves and their families.
- o Self-advocacy can be small, everyday choices or **publicly speaking out** about important issues like access to services.
- o Because of stigma and discrimination, some persons with disabilities may need **support from an advocate** (family, friend, OPD member, or community volunteer).
- o Advocates can help by **providing information, offering support in meetings, writing letters, or speaking on behalf of the person** in challenging situations.

6.2.2 What is communication?

Communication is the process of conveying messages, exchanging information, ideas, thoughts between individuals or groups so that understanding is achieved. It can occur through various channels including verbal (spoken or written language) nonverbal (sign language, body language gestures and facial expressions) and visual (images, symbols and graphs). Effective communication involves not only transmission of messages but also understanding and interpretation by the recipient hence giving feedback, only then communication is complete.

Elements of effective communication:

- o **The means:** refers to how a message is sent. This may occur via spoken or written words, sign language, and/or assistive communication devices. People require access to the means of communication that best suits their needs
- o **The reason:** refers to why communication takes place. The many reasons include: expression of practical needs, e.g. asking for a drink etc.
- o **The opportunity:** people need frequent opportunities to communicate with others. Many people with disabilities do not have the opportunity to communicate, e.g. children with disabilities are often hidden when visitors come to the family home, a deaf person is unable to participate in a council meeting because a sign language interpreter is not available etc.
- o **The response:** after a message has been sent, it is key as it shows the communication is complete or has reached the intended recipient and feedback provided.

Communication by persons with disabilities

Persons with disabilities can still express their likes and dislikes, wants and needs, in ways other than words. Communication also includes display of text, Braille, tactile communication, large print, accessible multimedia as well as written, audio, plain-language, human-reader and augmentative and alternative modes, means and formats of communication.

Communication barriers

There are four major barriers:

1. **Physical barriers**, e.g. long distances or environmental distractions,
2. **Barriers of meaning**, e.g. the use of complex words and specialized vocabulary,
3. **Attitudinal barriers**, e.g. avoidance of communicating with someone who has difficulty expressing her/himself, due to impatience and arrogance.
4. **Positioning of information** e.g. inappropriate timing or too much information.

For communication to be effective, it is important to identify those barriers which are present and take steps to overcome them.

Addressing communication barriers

CBID programs can help reduce or remove communication barriers by:

- o Encouraging positive attitudes in the home environment to maximize communication opportunities for children and/or adults with disabilities;
- o Identifying key people with whom persons with disabilities can communicate, and who are interested in what persons with disabilities have to say;
- o Sharing information with others about people's preferred means of communication; it is important to develop a means by which persons with disabilities can do this independently
- o Teaching family and community members' communication strategies that may be useful for persons with disabilities, e.g. allowing extra time for communication, and using simple language, gestures, signs, drawings or braille;
- o Providing suggestions about how the environment might be adapted to maximize communication, e.g. a quiet space to talk for a person with a hearing impairment, adequate light for a person who lip-reads;
- o Promoting the development of a pool of trained sign language interpreters – it may be helpful to partner with organizations of and for persons with disabilities to do this;
- o Using public meetings or the media to increase community awareness about the challenges faced by persons with communication problems;
- o Together with organizations of and for persons with disabilities, promoting the need for and use of accessible information and communication formats in all activities, such as training, community events and development programs.

Key Takeaways – Communication

- o Communication is complete only when messages are understood and responded to.
- o Persons with disabilities may use multiple forms of communication (e.g., Braille, sign language, large print, assistive devices, gestures).
- o Effective communication depends on:
 - The means (how the message is sent),
 - The reason (why communication happens),
 - The opportunity (having chances to communicate),
 - The response (feedback showing the message was received).
 -
- o Major barriers include physical, language/meaning, attitudinal, and timing/overload of information.
- o CBID can help reduce barriers by encouraging positive attitudes, adapting environments, shar-

ing preferred methods, training families/communities, and promoting accessible formats and interpreters.

Essence: Communication is a right, removing barriers and creating opportunities ensures persons with disabilities are heard and can participate fully.

6.3 Organizations of and for Persons with Disabilities

Module outcome

By the end of the session the learner should be able to: -

- Define Organizations of and for Persons with Disabilities.
- Explain the characteristics of OPDs
- Outline membership of OPDs
- Discuss ways in which CBID can involve OPDs
- Illustrate ways in which CBID program can support OPDs

Facilitators Notes

- **Step 1:** The Facilitator divides learners into small groups to define the concepts.
- **Step 2:** Group presentation on concepts
- **Step 3:** The Facilitator asks learners to brainstorm on characteristics of Disabled persons organizations and how they are organized.
- **Step 4:** The Facilitator asks the learners to brainstorm on how the OPDs can be supported by CBID program

Organizations of and for persons with Disabilities

An Organization of Persons with Disabilities (OPD). OPD is a representative body that is founded, led, and governed primarily by persons with disabilities themselves. OPDs are established to promote, protect, and advocate for the human rights, inclusion, and participation of persons with disabilities in all aspects of life.

An Organization for Persons with Disabilities is a body or institution that works on behalf of, or provides services to, persons with disabilities, but is not necessarily led or governed by them. These organizations may include organization that focus on focus on rehabilitation, education, health, social support or other services for persons with disabilities.

Characteristics of OPDs

- Led and Governed by Persons with Disabilities
- Grassroots and Membership-Driven
- Advocacy and Policy Influence
- Capacity Building and Empowerment Role
- Represent the Diversity of Disability
- Rights-Based Orientation

Membership of Organizations of Persons with Disabilities.

Membership of OPDs (Organization of and for Persons with Disabilities) consist of Individuals, persons with disabilities, their families, advocates, and other relevant stakeholders.

Involvement of OPDs

OPDs can support CBID by:

- o Promoting CBID at National and International level as a strategy to mainstream development and ensure the benefits of the Convention on the Rights of Persons with disabilities (CRPD) and the Sustainable Development Goals reach more persons with disabilities.
- o Lobbying for the inclusion of the CBID strategy in government policies and programs and necessary budgetary support.
- o Mobilizing communities by raising awareness about disability and the importance of CBID. Encouraging the formation of community groups for persons with disabilities in urban slums, rural villages, and assisting in building the capacities. Directly implementing CBID programs using the CBID matrix.

Support the formation of OPDs.

The strength of CBID programs is that they have direct contact with persons with disabilities at the grass root level and therefore can play a supportive role for persons with disabilities who are interested in forming OPDs.

CBID personnel can play a vital role in supporting their efforts by;

1. Identifying persons with disabilities who are interested in forming their own organization and ensuring there is representation from women and children with disabilities, all disability groups and people from different socio-economic groups.
2. Providing information about the benefits of forming an OPD.
3. Working in partnership with OPDs at the National and Regional level to organize training workshops to build the capacity of potential local members of OPDs.
4. Assisting them to develop their organizational structure, mission, objectives and plan of action, register with the local authorities, open bank accounts and complete any other necessary legal formalities.
5. Providing ongoing assistance once the OPD is formed.

Key Takeaways.

- o **OPDs (Organizations of and for Persons with Disabilities)** give persons with disabilities a **collective voice** to advocate for their rights and influence decision-makers.
- o They help **break barriers of exclusion** by uniting persons with disabilities, their families, and allies.
- o OPDs support **CBID** by promoting disability inclusion in policies, mobilizing communities, raising awareness, and building self-help groups.
- o CBID staff can **support new OPDs** by identifying potential members, ensuring diverse representation, guiding on registration and structure, and providing ongoing capacity building.

In essence: OPDs are critical because they empower persons with disabilities to organize, advocate, and participate equally in society.

6.4 Community Mobilization

Module outcome

By the end of the session the learner should be able to: -

- o Define community and community mobilization
- o Discuss disability as a community issue
- o Illustrate how to create awareness on disability issues in the community.

Facilitators Notes

- **Step 1.** The facilitator asks learners to brainstorm on the meaning of concepts,
- **Step 2.** The facilitator provides input on the definition of concepts.
- **Step 3.** The facilitator divides learners into small groups to discuss how disability is a community issue. And creation of awareness in the community.
- **Step 4.** The Facilitator provides input on disability as a community issue.
- **Step 5.** The Facilitator provides input on awareness creation on disability issues in the community .

Community

A “community” can be described as a group of people living together or share common interest, values, goals or identity.

However, generally communities are not homogeneous, and their members often have different political, economic, social and cultural characteristics, interests and aspirations

Community mobilization

Community mobilization is the process of bringing together as many stakeholders as possible to raise people’s awareness of and demand for a particular program to assist in the delivery of resources and services, and to strengthen community participation for sustainability and self-reliance.

Disability as a community issue

Community development issues that relate to health, education, water and sanitation, housing, transportation and the environment, all have an impact on the incidence and prevalence of disability. In many communities there are barriers which impact on the quality of life of people with disabilities and their family members. These include physical/environmental, attitudinal, cultural, and services, system and policy barriers.

When the CBID program involves communities in their work it is more likely that the benefits for persons with disabilities and their family members will continue even after funding and support may have ended.

Awareness creation in the community

To successfully mobilize a community and gain support for disability initiatives, community members must first develop awareness about disability and begin to understand why it matters and what action they can take. Many community members will have limited knowledge about disability and as a result will have negative attitudes and behaviors towards persons with disabilities; disability may merely be perceived as a health issue, and approached with pity and stigma. There are many different ways to teach and create awareness about disability in the community. For example, CBID Programmes could use group discussions, role-plays, flashcards, storytelling, songs, drama, puppet shows, posters, films and radio. There

are important things to remember when raising awareness about disability in the community, such as:

- Messages must be simple;
- The methods used should be appropriate to the local culture;
- Awareness- raising activities will have a greater impact when persons with disabilities are directly involved in implementing them;
- Attitudes and behaviors take time to change and is an ongoing process

Key Take-aways – Section One: Community

- Communities are diverse, with different social, economic, and cultural dynamics.
- Community mobilization brings people and stakeholders together for shared action, strengthening participation and sustainability.
- Disability is a community issue, barriers in health, education, environment, attitudes, and policies affect persons with disabilities and their families.
- Awareness creation is crucial for changing perceptions; it works best when messages are simple, culturally appropriate, and when persons with disabilities are actively involved.
- Changing attitudes is a gradual process that requires ongoing effort.

In essence: Building inclusive communities requires awareness, participation, and collective responsibility to break barriers and sustain benefits for persons with disabilities.

6.5 Community Groups

Module outcome

By the end of the session the learner should be able to: -

- Define Community groups
- Discuss characteristics of Community groups
- Discuss membership of Community groups
- Outline Challenges of organizing Community groups
- Discuss Support required to persons with disabilities to form community groups

Facilitators Notes

1. The Facilitator in plenary asks learners to brainstorm on the definition of Community groups
2. The Facilitator provides input.
3. The Facilitator divides learners into small groups to identify and discuss characteristics of Community groups
4. Group presentations in plenary and Facilitator provides input.
5. In small groups Facilitator asks learners to identify and discuss challenges of organizing Community groups and support required by persons with disabilities in forming groups
6. Participants present in plenary and Facilitator provides input

6.5.1 Community groups

What are **Community** groups?

Community Group means a voluntary association of individuals from the same community which is self-organized for a common purpose aimed at improving the livelihood of the group members or for a community benefit and includes a special interest group, community project and community-based

organization but shall not include a Public Benefit Organization, Non-Governmental Organization or groups formed to champion a political cause or contrary to public policy

Community groups can serve many different purposes depending on the situation and the need. For example, within the development sector, community groups have been used as an effective strategy for poverty alleviation, human development and social empowerment.

Characteristics Community Groups

Some common characteristics of community groups that are associated with CBID Programmes includes:

1. Voluntary nature – they are run by and for group members, have regular meetings, and are open to new members
2. Generally being formed in response to a particular issue, e.g. no access to education for children with disabilities, limited income-generating opportunities;
3. Clear goals, which originate from the needs of group members and are known and shared by all members.
4. Informal structure and basic rules, regulations and guidelines to show members how to work effectively together;
5. Participatory nature – involving getting help, sharing knowledge and experience, giving help, and learning to help oneself.
6. Shared responsibility among group members – each member has a clear role and contributes his/her share of resources to the group;
7. Democratic decision-making;
8. Governance by members, using an external facilitator only, if necessary, in the formation of the group.
9. Evolution over time to address a broader range of issues;
10. Possibility of joining together to form a CBO of groups across a wider area.

6.5.2 Membership Community Groups.

Group members are volunteers, in the sense that they are not paid, but they work systematically and regularly to change their own situation through mutual support. Within the context of CBID, community groups usually comprise persons with disabilities and their families.

These groups are usually small, consisting of only a few people, but over time may grow and include 5–30 members. Small groups enable effective participation of all members in discussions and decision-making, whereas larger groups may have more power and influence.

Challenges of organizing Community Groups

Rural vs. urban Community Groups

CBID experience shows that sustainability of community groups in urban settings can be particularly challenging because of frequent migration and difficulties in building trust and a sense of belonging among group members. Although it should be noted that, in rural areas, geographical isolation and the long distances group members may need to travel, and the limited means of communication, may make it difficult to hold regular meetings.

Women and men

The CBID experience indicates that women's groups are generally easier to form than groups for men. Women tend to have a stronger sense of solidarity towards one another and work more easily in a

collaborative way. Where groups have both male and female members, it is important to ensure that women are represented, their voices heard and their issues discussed.

Levels of education

Within community groups, members may have varying levels of education. It is likely that persons with disabilities will have lower levels of education and therefore may be disadvantaged. It is important that meetings are not monopolized by those people who may have a higher level of education. To prevent inequalities from arising within groups, time needs to be dedicated in the early stages of group formation to building a sense of cohesion and empowerment among the less literate or vocal members.

Single-disability groups

Many groups have been formed for and by individuals with similar disabilities. While single disability groups have a clear and well-defined purpose, often cross-disability groups are more practical in small communities where there are few people with one particular type of disability. The basic needs of persons with disabilities, irrespective of their impairments, are the same, e.g. food, shelter, health care, education. Single- disability groups can often divide persons with disabilities and set up competition for scarce resources.

Dependency

Disability is often associated with dependency, e.g. with doing things for people with disabilities rather than doing things with them. As a result, persons with disabilities who are used to being recipients rather than contributors may lack the motivation and confidence to participate in self-help groups and activities.

6.5.3 Support required to persons with disabilities to form community groups.

Mobilization

Provide information to persons with disabilities and their family members about the concept of community groups and encourage them to form a group in their community. This may include discussing the advantages of being in a group and the types of concerns that can be addressed.

Planning

Discuss the shared concerns and establish the initial focus of the group. This includes identifying necessary resources, member contributions, leadership roles, names of members, the group's name and the meeting arrangements.

Running the meetings

While CBID personnel are encouraged not to be in charge of running meetings, they need to be aware of the basic structure for a community group meeting so they can support and assist groups where necessary.

Support can be:

1. Facilitating
2. Building capacity
3. Develop partnerships with existing self-help groups
4. Encourage inclusion of persons with disabilities in mainstream self-help groups
5. Encourage self-help groups to join together to stronger

Key Take-away – Community Groups

- o Community groups are built on **mutual support**, not charity.
- o They are **voluntary, participatory, democratic**, and evolve to meet members' needs.
- o Membership is usually small (5–30), often including persons with disabilities and families, ensur-

ing active participation.

- o Community groups empower members through **shared responsibility, decision-making, and collective action**.
- o **Challenges include:** rural isolation, urban migration, gender dynamics, education gaps, dependency attitudes, and fragmentation from single-disability groups.
- o **Success requires:** mobilization, planning, inclusive leadership, capacity-building, and linking with other Community groups or mainstream groups.

In essence: Community Groups are a practical way to build solidarity, confidence, and collective power among persons with disabilities and their families, enabling them to address shared challenges and drive change together.

6.6 Political Participation

Module outcome

By the end of the session the learner should be able to: -

- o To define political participation
- o To explain the importance of political participation in the community
- o To identify barriers to political participation
- o Participation of persons with disability

Facilitators Notes

- **Step 1;** The Facilitator introduces the topic and gives the objectives.
- **Step 2;** Facilitator asks the learners to present a role play on the importance of political participation
- **Step 3;** Facilitator asks learners to brainstorm on the barriers to political participation.
- **Step 4;** The Facilitator divides learners into small groups to define the concepts
- **Step 5;** Group presentations on concepts
- **Step 6;** Facilitator wraps up by providing input

6.6.1 What is Political participation?

Political participation refers to the engagement of citizens in the political process, encompassing various activities that influence or shape political outcomes. It is a fundamental aspect of democracy, as it allows individuals to have a voice in the governance of their communities, regions, or countries. Including persons with disabilities in political participation involves creating an inclusive environment that accommodates their diverse needs and ensures their voices are heard.

Promoting the participation of persons with disabilities in politics is essential for empowering individuals, advancing disability rights, and building more inclusive and democratic societies. By removing barriers to political engagement, fostering leadership opportunities, and amplifying the voices of persons with disabilities, societies can create a more equitable and representative political system that benefits everyone.

The roles of CBID

The roles of CBID are:

1. **Empowerment:** CBID programs empower persons with disabilities by providing them with knowledge, skills, and resources to assert their rights and participate actively in political affairs. Through education, training, and capacity-building initiatives, persons with disabilities gain confidence, self-esteem, and a sense of agency, enabling them to engage more effectively in

political processes.

- 2. Meaningful Participation:** CBID promotes inclusive participation by breaking down barriers to access and ensuring that persons with disabilities are included in community activities, decision-making processes, and leadership roles. By fostering an environment of acceptance, respect, and support, CBR programs create opportunities for persons with disabilities to contribute their talents, expertise, and perspectives to community development efforts and political discourse.
- 3. Awareness and Sensitization:** CBID initiatives raise awareness about disability rights, social inclusion, and the importance of political participation among community members, local leaders, and policymakers. By challenging stereotypes, addressing stigma, and promoting positive attitudes towards persons with disabilities, CBID programs create a more supportive and inclusive environment for political engagement.
- 4. Capacity Building:** CBID builds the capacity of local organizations, community leaders, and stakeholders to support the political participation of people with disabilities effectively. By providing training on disability rights, accessibility, advocacy strategies, and inclusive practices, CBID empowers communities to create inclusive policies, programs, and services that address the needs of individuals with disabilities.
- 5. Advocacy and Lobbying:** CBID organizations advocate for the rights of persons with disabilities at the local, national, and international levels, lobbying for policy reforms, legal protections, and resource allocations that promote political participation and social inclusion. By mobilizing grassroots support, conducting awareness campaigns, and engaging with decision-makers, CBID initiatives amplify the voices of people with disabilities and influence political agendas.
- 6. Networking and Collaboration:** CBID fosters collaboration and networking among disability organizations, civil society groups, government agencies, and other stakeholders to strengthen collective efforts to promote political participation and disability rights. By building alliances, sharing best practices, and coordinating advocacy efforts, CBID initiatives leverage collective resources and expertise to drive systemic change and advance the rights of persons with disabilities.

6.6.2 Barriers to political participation

- 1. Physical Barriers:** Physical barriers such as inaccessible polling stations, lack of transportation, and inadequate facilities can prevent persons with disabilities from accessing political events, voting booths, or community meetings. Lack of ramps, elevators, or appropriate signage can further restrict their ability to participate in civic activities.
- 2. Communication and information Barriers:** This includes lack of sign language interpreters, captioning, or assistive communication devices, limited access to information and voter education materials in accessible formats, such as braille, large print, or audio recordings can hinder persons with disabilities from engaging in political discourse, attending public forums, or accessing information about political candidates and platforms.
- 3. Attitudinal Barriers:** Negative attitudes, stereotypes, and misconceptions about persons with disabilities may lead to social exclusion, discrimination, or marginalization, affecting their confidence, self-esteem, and willingness to participate in political activities. These attitudes may be perpetuated by both individuals and institutions, including political parties, government agencies, and the media.

- 4. Legal Barriers:** Legal barriers, including discriminatory laws, regulations, or voting procedures, may restrict the political rights and participation of persons with disabilities. Lack of legal protections, accommodations, or enforcement mechanisms can undermine their ability to exercise their right to vote, run for office, or engage in political advocacy.
- 5. Economic Barriers:** Economic barriers, including poverty, lack of financial resources, or limited access to education and employment opportunities, can create challenges for persons with disabilities to actively participate in political activities or campaigns. Lack of funding for disability organizations or support services may further exacerbate these barriers.
- 6. Socio – cultural barriers:** Social barriers, such as stigma, prejudice, and social isolation, can impact persons with disabilities' ability to build social networks, form alliances, or mobilize support for political causes. Limited opportunities for socialization, networking, or community engagement may hinder their ability to connect with peers, allies, or mentors in the political arena.

6.6.3 Suggested activities

To ensure that Community-Based Inclusive Development (CBID) personnel develop awareness of the political system, the following strategies can be employed:

- 1. Training and Education:** Provide training sessions, workshops, and educational programs to CBID personnel on the political system, including its structures, processes, institutions, and key stakeholders. This training should cover topics such as governance, democracy, citizenship rights, electoral systems, and the role of government in disability inclusion.
- 2. Contextual Analysis:** Help CBID personnel understand the political context in which they operate, including the political landscape, power dynamics, and decision-making processes at the local, national, and international levels. This involves analyzing political trends, policies, and priorities that impact disability rights and inclusion.
- 3. Policy Analysis:** Equip CBID personnel with skills in policy analysis and advocacy to engage with the political system effectively. This includes understanding how policies are formulated, implemented, and evaluated, as well as identifying opportunities for influencing policy change to advance disability rights and inclusion.
- 4. Stakeholder Engagement:** Facilitate interactions and dialogues between CBID personnel and political stakeholders, including government officials, policymakers, elected representatives, and civil society organizations. Encourage CBID personnel to participate in relevant meetings, forums, and consultations to advocate for disability-inclusive policies and programs.
- 5. Community Mobilization:** Empower CBID personnel to mobilize and engage with their communities around political issues related to disability rights and inclusion. This involves raising awareness, facilitating discussions, and organizing grassroots campaigns to promote civic engagement, voter education, and advocacy for policy change.
- 6. Networking and Collaboration:** Encourage CBID personnel to collaborate with other organizations and networks working on political participation and disability rights. This includes partnering with disability advocacy groups, human rights organizations, and electoral watchdogs to amplify their voices, share resources, and coordinate advocacy efforts.

- 7. Monitoring and Reporting:** Support CBID personnel in monitoring political developments, policy changes, and legislative reforms relevant to disability rights and inclusion. Encourage them to document their observations, experiences, and challenges in engaging with the political system and to report back to their organizations and communities.
- 8. Continued Learning and Reflection:** Foster a culture of continued learning and reflection among CBID personnel, encouraging them to critically analyze their experiences, strategies, and outcomes in engaging with the political system. Provide opportunities for peer learning, mentoring, and professional development to enhance their political awareness and effectiveness.

6.6.4 Facilitate access to political processes

- 1. Accessible Information:** Provide accessible information about political processes, candidates, and platforms to persons with disabilities. This includes producing materials in various formats such as braille, large print, audio recordings, and easy-to-read formats. Ensure that information is available through accessible websites and digital platforms.
- 2. Accessible Voting:** Ensure that polling stations and voting procedures are accessible to persons with disabilities. This includes providing physical accessibility features such as ramps, handrails, and accessible parking spaces, as well as providing assistive technologies such as magnifiers, braille templates, and tactile ballot guides. Train polling staff to assist voters with disabilities effectively and respectfully.
- 3. Voter Registration:** Make voter registration processes accessible and inclusive for persons with disabilities. Provide alternative registration methods such as online registration, home visits, and mobile registration units. Ensure that registration materials are available in accessible formats and that registration venues are fully accessible.
- 4. Voter Education:** Conduct voter education campaigns targeted at persons with disabilities to ensure they understand their rights and responsibilities in the electoral process. Provide information about how to register to vote, where and when to vote, and how to access assistance or accommodations at polling stations.
- 5. Support Services:** Offer support services and accommodations to persons with disabilities to facilitate their participation in political processes. This may include providing sign language interpreters, personal assistants, accessible transportation, and assistance with completing ballots for voters with visual or cognitive impairments.
- 6. Training and Capacity Building:** Provide training and capacity-building programs for persons with disabilities to empower them to engage effectively in political processes. Offer workshops on advocacy skills, leadership development, public speaking, and campaign management tailored to the needs of persons with disabilities.
- 7. Legal Protections:** Ensure that laws and regulations protect the rights of persons with disabilities to participate fully in political processes. Enact legislation that guarantees accessibility, non-discrimination, and reasonable accommodations in electoral laws and procedures. Establish mechanisms for addressing complaints of discrimination or barriers to access in political participation.
- 8. Community Outreach:** Conduct outreach activities to engage persons with disabilities and raise awareness about the importance of political participation. Collaborate with disability organizations, community groups, and local leaders to organize events, forums, and discussions.

on disability rights and political inclusion.

- 9. Monitoring and Evaluation:** Monitor the accessibility and inclusiveness of political processes for persons with disabilities and evaluate the effectiveness of efforts to facilitate access. Collect data on voter turnout, registration rates, and satisfaction with accessibility measures to identify areas for improvement and ensure continuous progress.

Key Take-away – Political Participation.

- o Persons with disabilities have the *right* to participate fully in politics and governance.
- o CBID strengthens this by:
 - Empowering persons with disabilities to engage confidently.
 - Advocating for accessible political spaces and processes.
 - Raising community awareness to break stereotypes.
- **Common barriers:** inaccessible polling stations, lack of sign language or Braille, stigma, and discriminatory laws.
- **Practical actions:** build leadership skills, provide accessible voter education, and lobby for inclusive policies.

In essence: Political participation is about moving beyond voting it's ensuring persons with disabilities can influence decisions, hold leadership roles, and shape governance on equal terms.

CROSS CUTTING INCLUSION PRINCIPLES

These key principles must be considered across all CBID components and modules to ensure the full participation, equality, and empowerment of persons with disabilities.

Inclusive community development requires that all activities, services, and spaces be accessible to persons with disabilities. This involves removing barriers, physical, communication, social, and attitudinal, that limit participation.

Applying universal design principles ensures that environments, tools, and information are usable by everyone, not only those with disabilities, making inclusion sustainable and practical.

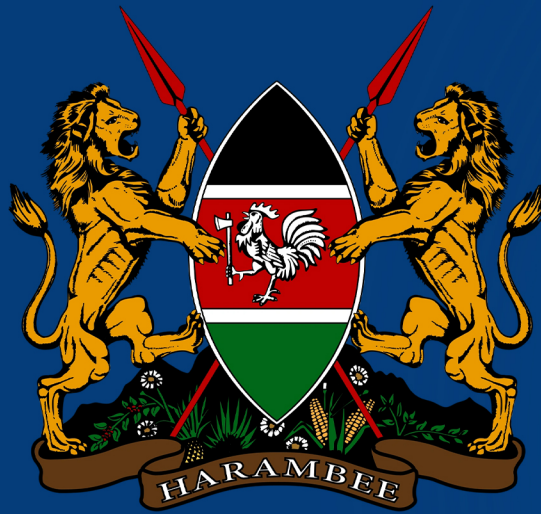
Reasonable accommodation refers to the adjustments or supports provided to enable persons with disabilities to participate on an equal basis with others, such as ramps, accessible transport, adaptive equipment, sign language interpretation, braille, or simplified formats.

All CBID initiatives should be guided by a human rights-based approach, which emphasizes equality, non-discrimination, and the active participation of persons with disabilities in planning, implementing, and monitoring programs.

By embedding these principles across health, education, livelihood, social, and empowerment activities, CBID ensures that communities are inclusive, equitable, and responsive to the needs of all members.

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